Ekagra Karma ✻ Apara Antara

INTEGRATED CLINICAL HYPNOTHERAPY FOUNDATION

Formerly - California Hypnosis Institute of India

ADVANCED INTEGRATED HYPNOTIC MODALITIES FOR HEALTH RESOLUTIONS (THEORY)

LEVEL III

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AIMS AND OBJECTIVES

At the end of this course the participants shall be able to:

- Determine the Physical & Emotional Sexuality of a client
- Provide Relationship Counseling
- Use Family Systems Approach to understand the issues of a client
- Understand the issues of Adult Children of Dysfunctional Families
- Understanding the need for Child Hypnosis
- Carry out First Consultation
- Use Counseling & Interviewing techniques
- Learn the theory of Addressing the following disorders:
  - Eating Disorders
  - Passive Aggressive Behavior (PAB)
  - Detrimental habits- Nail biting, Tardiness etc.
  - Sexual Dysfunctions
- Understand the basics of Handwriting Analysis (Graphology)
- Understand the basics of Past Life Regression
- Use of Mental Bank Technique

PROGRAMME SCHEDULE

5 DAY –PROGRAMME			
Day 1			
From	**To**	**Duration in mins.**	**Contents**
0900	0930	30	Registration & Introductions
0930	0945	15	Questionnaire
0945	1030	45	The “Why” of relationships
1030	1045	15	**Tea Break**
1045	1130	45	Physical and Emotional Sexuality
1130	1200	30	How sexuality is developed?
1230	1300	30	P&E Male & Female Traits
1300	1330	30	P&E Male & Female Relationships
1330	1430	60	**Lunch**
1430	1500	30	Honeymoon Stage & Familiarity
1500	1530	30	Priorities for P&E Sexuals
1530	1545	15	**Tea Break**
1545	1615	30	Sex and Cycle Days for P&E
1615	1630	15	Rejection & Complementarity
1630	1645	15	Core Traits
1645	1715	30	*Discussion of Sexuality Questionnaires results*
1715	1800	45	Relationship Counseling & Negotiation

5 DAY –PROGRAMME			
Day 2			
From	**to**	**Duration in mins**	**Contents**
900	930	30	Brief History of Family Systems
930	1000	30	Family Systems Definitions
1000	1015	15	Types of Dysfunctions and Functional Family Systems
1015	1045	30	**Tea Break**
1045	1115	30	Traits of Dysfunctional and Healthy Family Systems
1115	1145	30	Roles of Children in Dysfunctional Families & Physical and Sexual Abuse of Children
1145	1200	15	Problems with Adults from Dysfunctional Families
1200	1215	15	Co-Dependency / Co-Dependents in Therapy
1215	1230	15	Other High-Stress Family Systems
1230	1330	60	**LUNCH**
1330	1400	30	Cord Cutting Theory
1400	1500	60	Inner Child Theory
1500	1545	45	Child Hypnosis
1545	1615	30	Enuresis
1615	1630	15	**Tea Break**
1630	1700	30	Suggested Schedule for First Consultation and getting people in the Mood for Change
1700	1730	30	Different approaches to interviewing & Avoidance Reactions
1730	1800	30	Wrap & Closure

5 DAY –PROGRAMME			
Day 3			
From	**To**	**Duration in mins.**	**Contents**
0900	0945	45	Eating Disorders, Anorexia Nervosa/Bulimia and Compulsive Eating
0945	1015	30	Habit Control - Nail Biting, Tardiness, Weight Control, Study Habits & Examination Anxiety
1015	1115	30	Causes for Sexual Dysfunction, Guidelines for Couple Therapy and Premature Ejaculation, Impotence and Sexual Response Cycle
1115	1130	15	**Tea Break**
1130	1200	30	What is a Crisis? And Explosive Personality Disorder
1200	1245	45	Wife Beating and Child Abuse; Incest & Sexual Child Abuse, Rape Victims
1245	1330	45	**LUNCH**
1330	1400	30	Freud‟s Model of the Mind – Id, Ego, Superego
1400	1500	60	Defense Mechanisms – Types and Common Characteristics
1500	1530	30	Passive Aggressive Behaviour - Theory
1530	1600	30	Void Identification – Theory
1600	1615	15	**Tea Break**
1615	1700	45	Handwriting Analysis (Graphology)
1700	1800	60	Past Life Regression – Theory

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ADVANCED INTEGRATED HYPNOTIC MODALITIES FOR HEALTH RESOLUTIONS

5 DAY –PROGRAMME			
Day 4			
From	**To**	**Duration in Mins**	**Contents**
0900	1030	90	Theory of Mind applied to Mental Bank & The Mental Bank Process - Affirmations & Investments
1030	1045	15	**TEA BREAK**
1045	1145	60	Online Test
1145	1230	45	Test Evaluation
1230	1300	30	Certification & Wrap up

MALE SEXUALITY QUESTIONNAIRE - A

Please read instructions carefully before beginning

1) In answering the questionnaire, refer to your most recent relationship in which you lived with a partner for at least six months.
2) If this does not apply, refer to the most recent intimate relationship you have had.
3) Do not jump from relationship to relationship, but answer each question in reference to the same relationship.
4) Answer questions candidly, and in a way that you respond most of the time in that intimate relationship. Please take your time and answer these questions carefully.
5) If you feel a certain question does not apply, answer as best as you can.
6) Please leave no question unanswered.

	Questions	**Yes**	**No**
1	Answer yes if your parent(s) had any one or more of the traits listed. If you were raised by both parents (up to age 15 or 16), was your father more outward and demonstrative in showing affection for you than your mother (by hugging, verbalizing feelings, compliments etc.)?		
	If you were raised by your father only, was he outward and demonstrative in showing affection for you?		
	If you were raised by your mother only, was she outward and demonstrative in showing affection for you?		
2	Do you often desire repeated intercourse when having sex with your partner?		
3	After a disagreement with your partner, do you usually make up first?		
4	Are you more jealous or possessive of your partner than she is of you?		
5	Do you feel that having sex after an argument is a good way to make up?		
6	Do you like to show your partner attention by opening doors, helping her with her coat, pulling her chair out before she sits down, etc.?		
7	Immediately following the sex act, do you like to touch and caress your partner?		
8	If you feel that you"ve been unfairly criticized or rejected by your partner, are you capable of expressing extreme anger, tantrums, or vindictiveness?		
9	When you meet someone of the opposite sex, are you first attracted to the area of her body from the waist down rather than from the waist up?		
10	If you"ve been strongly rejected by your partner, do you feel actual physical discomfort or pain?		
11	Is it important for you to share most of your social activities and hobbies with your partner?		
12	If your partner breaks off a relationship that you do not want to end, do you find that all your energies and thoughts keep drifting back to her and you have difficulty concentrating on anything else?		
13	Do you agree with your partner at times to prevent her turning off sexually, even when you think she is wrong?		

14	Do you feel that you give more of yourself to your partner than she does to you?		
15	Do you like your partner to be more sexually aggressive and creative than she is?		
16	Do you enjoy receiving attention and flattery from your partner in the presence of others?		
17	Is the woman in your life your number one priority?		
18	If you suspected your partner of cheating, would you put the greatest degree of blame on the man for leading her astray?		
19	Do you feel that you have a greater capacity for love and deep emotional feelings than your partner?		
20	Does it disturb you if your partner‟s sex drive diminishes after the newness of the relationship wears off?		

MALE SEXUALITY QUESTIONNAIRE B

Please read instructions carefully before beginning

1) In answering the questionnaire, refer to your most recent relationship in which you lived with a partner for at least six months.
2) If this does not apply, refer to the most recent intimate relationship you have had.
3) Do not jump from relationship to relationship, but answer each question in reference to the same relationship.
4) Answer questions candidly, and in a way that you respond most of the time in that intimate relationship. Please take your time and answer these questions carefully.
5) If you feel a certain question does not apply, answer as best as you can.
6) Please leave no question unanswered.

Sr	Questions	Yes	No
1	Answer yes if your parent(s) had any one or more of the traits listed. If you were raised by both parents (up to age 15 or 16), was your father more passive and undemonstrative in showing affection for you than your mother (by hugging, verbalizing feelings, compliments etc.)?		
	If you were raised by your father only, was he undemonstrative, passive, cold, withdrawn, or overly strict?		
	If you were raised by your mother only, was she undemonstrative, passive, cold, withdrawn, or overly strict?		
2	Instead of complimenting your partner, do you usually take the attitude that as long as you do not complain, everything is OK?		
3	Is the expectation of sex often greater than the actual physical act?		
4	Do you feel that it is an unnecessary act to open doors and light cigarettes for a woman, even though you may do it?		
5	When you make up after a fight, do you still feel resentment and find it difficult to fully forgive?		
6	Does your partner want sex more often than you do?		
7	Do you dislike „wining and dining" a female in order to have sex, assuming that money is no object?		
8	Answer only one: a) If married, do you have or prefer to have an additional relationship outside your marriage?		
	b) If single, do you usually have one steady partner but date other people at the same time?		
9	Would you avoid or refuse to have sex with your partner after an argument?		
10	Would you rather avoid verbally expressing love, tenderness or affection immediately following intercourse?		
11	When you meet someone of the opposite sex, are you first attracted to the area of her body from the waist up rather than from the waist down?		
12	Do you turn off during sex if distracted by small talk or something that you feel criticizes you?		

13	Are you constantly searching outside of your relationship for the "romance" you feel is missing in your life?		
14	Do you find that as the newness of a relationship wears off your sex drive towards your partner diminishes?		
15	Would you have a strong resentment against a female becoming pregnant by you unless perhaps you are well established in a marriage and ready to have children?		
16	Do you have the attitude that if you felt the same way before the sex act that you feel five minutes after, you would never have sex?		
17	During the sex act, would you prefer to avoid verbally expressing the different physical and emotional feelings you are experiencing?		
18	During an argument, does your partner tend to throw back at you things you said or did in the past that hurt, angered, or rejected her?		
19	When you are having sex with your partner, do you often fantasize about someone else or about another sex act?		
20	Does it annoy you to have to reassure your partner by giving her compliments or attention that she outwardly solicits from you?		

FEMALE SEXUALITY QUESTIONNAIRE A

Please read instructions carefully before beginning

1) In answering the questionnaire, refer to your most recent relationship in which you lived with a partner for at least six months.
2) If this does not apply, refer to the most recent intimate relationship you have had.
3) Do not jump from relationship to relationship, but answer each question in reference to the same relationship.
4) Answer questions candidly, and in a way that you respond most of the time in that intimate relationship. Please take your time and answer these questions carefully.
5) If you feel a certain question does not apply, answer as best as you can.
6) Please leave no question unanswered.

Sr.	Questions	Yes	No
1	Answer yes if your parent(s) had any one or more of the traits listed.		
	If you were raised by both parents (up to age 15 or 16), was your father more outward and demonstrative in showing affection for you than your mother (by hugging, verbalizing feelings, compliments etc.)?		
	If you were raised by your father only, was he outward and demonstrative in showing affection for you?		
	If you were raised by your mother only, was she outward and demonstrative in showing affection for you?		
2 A B	Does “b” more closely describe what you usually feel at the culmination of the sex act than „a"? A sudden end to all pleasurable feelings (stimulation may turn to an irritation or ticklish frustration) and a feeling of wanting to back off and stop the sex act momentarily or completely. A physical and emotional release with contractions, spasmodic shivering, body warmth, moisture, and capabilities of multiple releases.		
3	If your partner breaks off a relationship that you don"t want to end, do you find that all your energies and thoughts keep drifting back to him, and that you are unable to concentrate on anything else?		
4	Immediately following the sex act, do you enjoy being physically close, touched and caressed by your partner?		
5	During an argument, do you tend to throw back at your partner things he has said in the past that hurt you?		
6	After a disagreement with your partner, do you usually make up first?		
7	Are you more jealous or possessive of your partner than he is of you?		
8	Do you often desire repeated or prolonged intercourse when having sex with your partner?		
9	Is it important to you to have your partner show you attention in public by opening doors, helping you with your coat, pulling out your chair before you sit down, etc.?		
10	During intercourse, do you verbally express the different emotional		

	feelings you experience within you?		
11	If rejected by your partner, are you capable of expressing extreme anger, tantrums, or physical violence?		
12	Do you enjoy buying gifts for your partner?		
13	Do you have a tendency to walk and stand with your feet pointed more outward than inward?		
14	Are you still willing to have sex with your partner even after arguments, rejections, or periods of neglect?		
15	Do you enjoy receiving outward affection and flattery from your partner?		
16	Does your sex drive seem to be stronger than your partner"s?		
17	Do you feel that you are better able to express intimate feelings and attitudes than your partner?		
18	Do you feel that you put more energy into making your relationship work than your partner does?		
19	Do you agree with your partner at times to prevent him from turning off sexually, even when you know he is wrong		
20	Do you feel that one of the ultimate fulfillments of womanhood is to have your own children?		

FEMALE SEXUALITY QUESTIONNAIRE B

Please read instructions carefully before beginning

1) In answering the questionnaire, refer to your most recent relationship in which you lived with a partner for at least six months.
2) If this does not apply, refer to the most recent intimate relationship you have had.
3) Do not jump from relationship to relationship, but answer each question in reference to the same relationship.
4) Answer questions candidly, and in a way that you respond most of the time in that intimate relationship. Please take your time and answer these questions carefully.
5) If you feel a certain question does not apply, answer as best as you can.
6) Please leave no question unanswered.

Sr.	Questions	Yes	No
1	Answer yes if your parent(s) had any one or more of the traits listed.		
	If you were raised by both parents (up to age 15 or 16), was your father more passive and undemonstrative in showing affection for you than your mother (by hugging, verbalizing feelings, compliments etc.)?		
	If you were raised by your father only, was he undemonstrative, passive, cold, withdrawn, or overly strict?		
	If you were raised by your mother only, was she undemonstrative, passive, cold, withdrawn, or overly strict?		
2 A	Does “a” more closely describe what you usually feel at the culmination of the sex act than “b”? A sudden end to all pleasurable feelings (stimulation may turn to an irritation or ticklish frustration) and a feeling of wanting to back off and stop the sex act momentarily or completely.		
B	A physical and emotional release with contractions, spasmodic shivering, body warmth, moisture, and capabilities of multiple releases.		
3	Would you feel satisfied sexually if your partner reaches climax or orgasm even if you did not?		
4	Would you prefer to avoid the use of verbal voyeurism (talking about or hearing the sex act described) during the sex act?		
5	Do you find that as the newness of a relationship wears off, your sex drive towards your partner diminishes?		
6	Would you rather stay at home than go out alone and seek contact with strangers?		
7	Is the expectation of sex often greater than the actual physical act?		
8	Do you have the attitude that you would never have sex if you felt the same way before the sex act that you feel five minutes after?		
9	Would you rather avoid verbally expressing love, tenderness, and affection immediately following intercourse?		
10	Do you turn off sexually during heavy kissing or heavy handling?		

11	Do your hands and feet usually feel colder than the rest of your body?		
12	Does prolonged or repeated intercourse turn you off?		
13	Are you constantly searching outside your relationship for the		
	“romance” you feel is missing in your life?		
14	Does your partner desire sex more often than you do?		
15	Do you turn off during sex if distracted by small talk or by		
	something that you feel criticizes you?		
16	Do you feel embarrassed or self-conscious if your partner		
	touches or handles you a great deal in public?		
17	Do you have a tendency to walk and stand with your feet toed		
	inward rather than outward?		
18	Would you avoid or refuse to have sex with your partner after an		
	argument?		
19	Instead of complimenting your partner, do you usually take the		
	attitude that as long as you don‟t complain everything is OK?		
20	Do you make excuses to avoid sex with your partner at times?		

SCORING INSTRUCTIONS FOR SEXUALITY QUESTIONNAIRES

- Count the number of "yes" answers to Questionnaire A and multiply by five.
- Do the same for Questionnaire B.
- Add the two scores together to obtain the combined score.
- Total of Sheet A divided by the sum total of A and B expressed as a percentage is your Physical sexuality score.

Note:

1. Once you have scored the Sexuality Questionnaires, refer back to the scores for the Suggestibility Questionnaires.
2. If you are Physically Sexual with more than 40% Emotional Suggestibility, add 10% to your degree of Physical Sexuality.
3. If you are Emotionally Sexual with more than 40% Physical Suggestibility, add 10% to your degree of Emotional Sexuality.
4. This is because Emotional Suggestibility has a tendency to suppress some areas of Physical Sexuality causing a person to appear less Physically Sexual than he or she really is.
5. Likewise, Physical Suggestibility decreases the manifestations of Emotional Sexuality causing a person to appear less Emotionally Sexual than he or she really is.

CHAPTER 1: - PHYSICAL & EMOTIONAL SEXUALITY

In olden times, it was assumed that the physical suggestible would also be a physical sexual and vice versa. Counselors also accepted it as the norm, in their attempt to understand behavioral science.

Dr. John Kappas realized that this behavior did not fit half the population. From his own experience and after many sessions of traditional relationship counseling, he found he did not like the “finger pointing” that was prevalent.

He understood that *suggestibility* and *sexuality* are two different things altogether. Suggestibility was defined as how we take things in and Sexuality was defined as how we put our ideas and what we learn, back-out to others.

In his therapeutic practice, he would see the exaggerated form of the sexual personality. People come into therapy at the point of break-up, which allows the therapist to see the clients in their worst behavior.

The important questions asked when determining sexuality were:
How did these two people ever get together in the first place?
And - How people function in the relationship?

On examining relationships – both his own and those of his clients, he began to see patterns. He then tested his theory and pointed it out to his clients in therapy with results which could be collated.

He then began to evolve his theory of physical and emotional sexuality.

THEORY OF DEVELOPMENT OF SEXUALITY:

Kappas‟s theory states that sexuality is primarily set up by the environment when the person is growing up.

At Stage one of its growth, the first person that the infant is in contact with is the mother, or the primary caretaker. In this stage, there is minimal or no recognition of sexuality as the child is the centre of the mother‟s world any which ways.

During stage two, the child is developing autonomy and also learns that there is a second person around, (generally of the opposite sex) i.e the secondary caretaker or a father figure. The child also realizes that the behaviour of the primary caretaker changes in the presence of the secondary caretaker and begins to model this father figure as a way to relate to the primary caretaker. *Sexuality therefore, is learned from the secondary caretaker.*

THE KEY POINT HERE IS NOT HOW FATHER ACTUALLY WAS, BUT HOW THE CHILD PERCEIVED THE FATHER FIGURE.

It is also important to analyze, how the secondary caretaker related to the child?
If the secondary caretaker was physical, then the child modeled physical sexuality (closeness, physical touching, etc.).
If the role model was not physical, then emotional sexuality developed and the child became comfortable in distance.

The questions to ask to see if this fits:
Male: What did my father teach me about women?
Female: What did my mother teach me about men?

Normally between the ages of thirteen and fifteen the sexuality is generally set and the Physical sets it quicker than the Emotional, for obvious reasons.

For the balance of this discussion, when discussing Physicals and Emotionals, the discussion will be about people with 100% Physical or Emotional sexuality.

When looking at people in between, they tend to understand more of the opposite sexuality, because they have more of the opposite sexuality in themselves.

The point is to understand that there is nothing right or wrong. Both behaviors are what they are, all we need to do is learn to understand and live with it, not try to change it. An emotional sexual should be called an intellectual. A physical sexual should be called an emotional.

Sexuality is a behavior, and as any behavior, it can be re-learned to be adapted to.

PHYSICAL & EMOTIONAL TRAITS:

FEMALE	
Physical Sexual	**Emotional Sexual**
Comfortable drawing attention to her body.	Not comfortable drawing attention to her body.
May do so by wearing very nice clothes, generally low-cut dresses, skimpy bathing suits, etc.	Will be more conservative and wear blouses buttoned all the way up.
May dress casual for a dressy occasion because it brings attention to her physical body.	Will dress appropriately for the occasion in order to merge with the crowd.
Will wear more stylish hairdos and have longer fingernails.	Will have practical hairdos and fingernails.

Male	
Physical Sexual	**Emotional Sexual**
Comfortable drawing attention to his body.	Not comfortable drawing attention to his body.
May do so by wearing an open shirt, jewelry, etc.	Will be more conservative and wear a suit and tie when appropriate.
Will tend to drive a sports car.	Will drive a more practical, conservative car.

The Physicals will brag about their accomplishments, and let you know that they want compliments.
The Emotional may want the compliment, but may turn it around when it is given, to nullify it.
Physicals are comfortable in groups; Emotionals are more comfortable in one-on-one situations.
At a party, the Physical will generally move from group to group, being a kind of "social butterfly", while the Emotional will sit in the corner, talking with one or two people.

PHYSICAL MALE & EMOTIONAL FEMALE

Typically, an Emotional female sees a Physical male as macho and confident, and assumes that he is strong emotionally. The Physical male will see the Emotional female as a "helpless female", someone he can take care of.

The problem occurs when the Physical male is found out to be insecure emotionally. He then tries to change the female. If the female is confident, she won"t take the suggestion to change. The Physical male then gets rejected, and doesn"t understand.

If the Emotional female is not confident, she will be put down by the suggestions, creating a one up – one down relationship.

EMOTIONAL MALE AND PHYSICAL FEMALE

They see traits in each other that they envy, so they are attracted to each other.
This happens when the emotions are brought out.

After some time, the female becomes very predictable to the Emotional male, always being there when he comes home at night, etc. When this happens, the Emotional male will go back to his workaholic ways, and the honeymoon stage of the relationship is over.

HONEYMOON STAGE

During the “Honeymoon Stage” of a relationship, the Emotional can act very Physical. Hence, the Emotional acts Physical and the Physical acts even more Physical.

The minute that the Physical says, “I love you”, the honeymoon stage is over, because the Emotional now has the Physical partner where they want them, in a commitment, which allows them to be Emotionals again.

This is an automatic subconscious response, or behavior. The Emotional thinks that now the relationship is all right, so they can go back to being a workaholic, because they let work slide to develop the relationship.

FAMILIARITY

As the relationship grows, so does familiarity. As it grows, the relationship becomes boring, unless some excitement is interjected into it.

This generally occurs somewhere during the seven year cycle (“seven year itch”). If the relationship is not working, the Emotional male will have affairs, generally with someone convenient, such as a secretary. Usually, the affair is with an Emotional female, who won‟t get bored when her lover talks about work.

If the wife finds out and expresses anger, the Emotional male will leave the relationship. If she reacts calmly, he will stay in the relationship out of guilt. The anger of his spouse will allow him to release his guilt.

With the Emotional female and Physical male, the Physical male will start more hobbies and get more into work when this occurs. The Physical male will not have an affair unless he wants to get caught (to get even).

The Emotional female may have affairs. She is not as fast to have an affair, and tends to have more guilt, than the Emotional male. However, she does have the ability to separate sex from love.

When the relationship ends, the Emotional is the one that pushed the Physical to end it. Very rarely does a Physical feel comfortable ending a relationship. When the relationship is over, the Emotional feels relief while the Physical feels that there is something wrong with him/her.

The Physicals operate from a premise of touch and affection. The Emotionals operate

from a premise of being the way they are because they want to be that way.

When the Physical has to leave the comfort zone of the relationship, they feel vulnerable and that something is wrong with them. They could not make love happen. A Physical may stay in denial for years.

A Physical sexual woman who is not finished with a past relationship will attract a Physical married man. It is a safe relationship.

An Emotional will move through the stages of loss more quickly. They adapt to change, and don"t tend to hang on to denial. Usually, the Emotional has replaced the relationship before they end the old relationship. This motivates them to get out of the old relationship and get on with the new. Or else, they would not have the courage to end it.

The Physical will become very picky in their next relationship. The Emotional will sit back and let someone come to them. They will be available for a Physical to get them.

The Physicals will continually put themselves in a position to be rejected. As a therapist, point this out to a Physical client. Begin to have them approach from the idea of "How can I get what I want by motivating the other person to give it to me?"

The Emotionals refuse to put themselves in a position to be rejected.

PRIORITIES FOR A PHYSICAL	**PRIORITIES FOR AN EMOTIONAL**
Relationship Children Friends and Hobbies Career The premise is acceptance through closeness with other people. A Physical"s identity comes from his/her relationship.	Career Hobbies Relationship and family Mistress and friendships The premises acceptance through accomplishment or achievement. An Emotional"s identity comes from his/her career.

There are three main reasons for the breakup of any relationship:

- Communication
- Sex
- Money

Problem Areas of Communication

- What we consciously know and say.
- What we know consciously and don"t say.
- What we don"t know consciously and don"t say, but are programmed to expect.

SEX:

In case of a relationship gone sour due to sex not being satisfactory as per the physical‟s perception, the physical feels rejected and may come in for counseling.

The first time two people go to bed together determines good or bad sex for the term of the relationship.

The Physical will initiate sex more so than the Emotional. The Emotional female is easier to get into bed than the Physical, because the Emotional does not want to create a confrontation by saying „No‟.

Also, the Emotional female can separate love and sex. With an Emotional and a Physical there are two totally different reasons for going to bed together, but they do not communicate these to the other.

The Physical is comfortable with physical expression. The Physical female is comfortable taking off her own clothes. The Emotional female wants to be undressed.

During sex, the Emotional narrows to the genitals for stimulation. The Physical feels all over his/her body.

The Physical will reach orgasm quicker than the Emotional because of body heat. (The Physical has a higher body temperature than the Emotional). The Physical can handle harsher touch than the Emotional. Emotionals need slow warm-up and a gentle touch.

In release, the Physical male can release many times a night, an Emotional generally just once.

Everyone has a series of sexual cubicles. The Physical will subconsciously only let go of part of what they have. This way, they can have repeated sex, and will hold back. A Physical will release only one cubicle at a time; an Emotional will release at least three cubicles at once.

Oral Sex:

An Emotional is comfortable with both giving and receiving oral sex; the Physical is not. The Physical feels more controlled during intercourse. Because the Physical concentrates on intercourse during sex, they don‟t enjoy oral sex.

The biggest area of missed communication is the area of sex. The Physicals are feeling so much that they assume their partner is feeling the same. Get the client to listen to the body language of their partner.

Goals in Sex:

The Physical‟s goal is to prolong sex because they enjoy it. For physicals in general sex is a need as they see it as a very important aspect of love and acceptance.

For the emotional it is a want or a desire, more often associated with release of stress, tension etc. Hence, an Emotional is primarily after release. An Emotional female will become irritated by prolonged intercourse.

CYCLE DAYS:

An Emotional is on a three-day cycle:
Day 1: No chance of sex
Day 2: Take it or leave it
Day 3: Highest time of stimulation

This may be exaggerated to seven or more days, depending on the individual.

A Physical is on a one-day cycle.

For men, the three-day cycle is fairly constant. The only thing that will change it is work.

For women, the fourth day after the period starts is a “3” or peak of stimulation. The next day, the cycle begins again.

The Physical likes attention and closeness after sex. The Emotional wants to just go to sleep or get on with something else afterward.

With two Emotionals or two Physicals together, there is no communication, because Physicals talk in inferences, while Emotionals talk literally.

A pair of Physicals will have a great sex life, while a pair of Emotionals will stare at the walls.

Exceptions

Remember, it is how the child *perceives* the secondary caretaker that determines the sexuality. If a Physical father is not there, the child will become an Emotional.

If an Emotional father decides to spend a great deal of time with a child, that child will become a Physical. By the same token, if a Physical father can‟t give affection to his child, that child will become an Emotional.

Defenses

Like in any therapy, treat / accept the client just the way they are acting or expressing, even if you know that they are using sexuality as a defense.

If you (the therapist) treat them the way they are acting, the defense will come down because the behavior is accepted.

Handwriting

Handwriting may give clues to true sexuality. Details in Chapter 13.

CHAPTER 2: - RELATIONSHIP COUNSELING

For any relationship to work, both partners must feel and know that they are accepted and understood irrespective of their traits.

CORE AND SURFACE TRAITS

As the name suggests, almost all people have certain innate or core traits which do not change and certain acquired or surface traits which may or may not change depending on the circumstances.

We as therapists understand our clients for Emotional or Physical sexuality by examining the core traits, not by surface traits.

There are three types of core traits:

- How people function in a relationship (sexual compatibility and complementarity).
- How people respond to rejection.
- How people relate to their parents.

COMPLEMENTARITY

This is what the two people did for each other to make the relationship work, how they complement each other to make a whole.

The patterns are just too strong to be random. People just don‟t pick each other. What we normally understand as “Sparks” and “Chemistry” is the essence of Complementarity.
It is important to realize that what one partner is weak in; the other will be strong in, and vice versa. When the weak fits with the strong, it makes one strong whole.

Example: Unconscious Reactions

Emotionals don‟t realize that they enjoy having Physicals make decisions for them. This is an unconscious reaction. The person is unaware why the chemistry was there in the first place.

REJECTION

When the same things that attracted the couple to come together in the beginning become a problem or are perceived as directed personally, the partners sense “rejection” and relationship begins to break apart.

The keyword to relationship counseling is *rejection*. Rejection normally meets with two types of responses with varying intensities:

- When rejection occurs, the person in the relationship runs.
- When rejection occurs, the person in the relationship holds on.

During the rejection phase of a relationship, the Physical will feel actual physical pain. The Emotional will feel it in the emotions but will work it out first, through denial, then through logic and reason.

When the relationship breaks down, what was a “perfect match” becomes what they most hate about each other. They then try to control the relationship and gain one-up-manship by dominating / manipulating what the partner is most sensitive about.

The Physical will get one up by trying to dominate career and hobbies of the Emotional. The Emotional will withdraw sex to gain control from a Physical.
This becomes a vicious cycle, ***domination vs. control.***

Relationship with parents:

Primary caretaker (mother): This is a forced relationship and is necessary for survival.

Secondary caretaker (father): In this relationship, anything done is done by choice.

Usually in the early stages of development, the child is jealous of the father‟s relationship with the mother. The child then models father to see how father “got” mother which is why sexuality comes from father, or secondary caretaker.

SURFACE TRAITS:

The surface traits accompany the core traits, but are not necessary for the person‟s sexuality.

How are traits acquired?

GENERATIONAL THEORY

This is the idea of looking back at previous generations of the same family, and mapping the sexuality.
Example: Emotional mother, Physical father have a daughter. She is a Physical, and marries an Emotional male. They have a son and a daughter, both of whom are Emotionals.

COUNSELING

When two people come in for counseling, one person is generally more committed to continuing the relationship than the other. (If both were committed to continuing, the couple would not be in therapy.)

People come in at the breaking point. The therapist should not use the same terminology for both partners simply because of differences in sexuality. It will make complete sense to only one partner.

The goal is to separate the two and deal with them separately.

An Emotional generally wants to end the relationship without guilt. A Physical wants to save the relationship.

Start making them look at the patterns. For the Emotional male, you could say something like “It‟s going to cost you money again.” With the Physical, be very literal and specific with instructions.

NEGOTIATION

Considering that both partners come for therapy.

- First ascertain a common goal and then through negotiation, work the clients toward the goal.
- One way to work this is through pacing (NLP). Tell them what they want to hear.
- Buy the symptoms:
- Get them to accept some of the patterns in their life.
- Most problems in the clients‟ lives arise due to their complete lack of comprehension of why the partner is behaving the way he/she is?
- Part of the therapeutic approach begins by the client understanding the rationale behind the partner‟s behavior.
- Explain sexuality and complementarity. Get the client to accept what they are (Emotional or Physical).
- Build back up to what they were when they fell in love. (This is for the Emotional.)

KNOWNS AND UNKNOWNS

Once the clients buy the symptoms, the unknowns become knowns. This lowers the message units, and makes the client less suggestible and sensitive. The behavior is less exaggerated, getting the client out of waking hypnosis. When this happens, the other person will see the good things, de-exaggerating the monster.

At the start of therapy, buy time with the Emotional with thoughts like “At least you‟ll know what to avoid next time.” With the Physical, tell them to give the Emotional space (this will be very hard for them to do).

RULES FOR RELATIONSHIP COUNSELING

The therapist is a mediator and a referee. This is to see that irreparable damage is not done. Rules must be set down as a safety valve so that there is nothing too painful and damaging that will come up.

Rules:

- Focus on the here and now, not on the past (there and then).
- Focus on “I” messages, not “you” or “they” messages. How does one know what the other feels? Not allowed to talk about how your partner feels, just how you feel.
- Focus on specifics, not generalizations. Statements with “always” and “never” are generalizations. Specifics can be changed, generalities cannot.
- No dumping and no bunching. Dumping: saving it all up and then dumping it on the mate. If they had eight to ten hours to deal with it and didn‟t, they should not expect their mate to deal with it in three minutes.
- Employ a feedback loop. This is a process of taking responsibility for your communication. “What does what your mate say mean to you?” Then, go back and say, “Is this what you mean / want?” Get them to be more specific! At first, get into specifics that can be dealt with.

Initially, rules start and apply only in the therapy session. Slowly they get extended to the outside world.

The reward principle:

This can best be described as giving in to the need of the other. With no reward, the problem grows and will smother you.

With an Emotional, the reward could be space. With a Physical, it could be sex, a kiss or a hug.

When the partner can‟t give the reward, the therapist must give it in hypnodrama. For a Physical, this must be a sexual hypnodrama. For an Emotional, it might be golfing, boating, working, etc.

Developing a contingency contract: When one does something for the other, the other member of the relationship must also give.

Teaching them to give a reward: Series of give and take between the two people. The therapist must negotiate the conditions for the reward.

CHAPTER 3: - FAMILY SYSTEMS APPROACH

FAMILY SYSTEM

A system can be defined as a set of units and the interrelationships between those units. In a family, the members are the units and their relationships with each other define the family system.

A family system can be either functional or dysfunctional, depending on the ways the members relate to each other and how the family as a whole is coping with relationship stressors and the challenges of everyday life.

In the 1950"s clinicians, such as Murray Bowen and Theodore Lidz, began to notice that hospitalized schizophrenics got worse after contact with their families. Psychoanalysts working with disturbed children noticed that parents often resisted growth-oriented changes in their children, sometimes removing them from treatment when improvements appeared.

The interpersonal relationships in the family became the unit of study and treatment, instead of an individual member who may have displayed symptoms for the entire family.

FAMILY SYSTEMS THEORY DEFINITIONS

Homeostasis refers to the self-maintenance of a system in balance or equilibrium. Functional and dysfunctional family systems attempt to return to their particular homeostasis. For example, when an alcoholic father stops drinking, the other members can make subtle attempts to get him drinking again. The family is brought to disequilibrium by changes in one member.

Identified Patient is the family member with the presenting symptoms. The identified patient is the member who is acting out the family pain and is often brought to treatment, or seeks treatment initially.

Complementary Relationship refers to a pattern of communication between people characterized by inequality and the maximization of differences. For example, a dominant husband and a submissive wife are in a complementary relationship.

Symmetrical Relationship is a pattern of communication between people characterized by equality and the minimization of differences. An example of a symmetrical relationship is a married couple that solves problems, negotiate, and treat each other as equals.

Triangling, according to Bowen, is the tendency of a two-person emotional system to recruit a third person (or a substance, hobby, excessive work, etc.) into the system when under stress. The triangulation is an attempt to alleviate the intensity of the stress between the two-person emotional system. An example could be - a child is often triangled into a parental dyad undergoing stress.

Differentiation of Self, in Bowen‟s view, is the goal of therapy. A differentiated person has a strong sense of self and a stable personal identity. He is able to say: "these are my opinions… this is who I am… this is what I will do, but not this… etc. He does not negotiate his sense of self or identity for the sake of others.

Undifferentiated Family Ego Mass, is Bowen‟s term for the intense interdependency (symbiosis) in a family. An individual sense of self fails to develop in members because of the existing fusion or emotional oneness (and family "we-ness").

Hierarchy refers to an arranged order of parts within the whole system. Parental hierarchy is the most common, wherein parents are in the position of decision making and establishing family rules.

Double Bind Messages are a set of contradictory messages from the same person to which an individual must respond, although his/her failure to please is inevitable whatever the response may be. Double Bind Messages create ambivalence, which is defined as a co-existence of two opposing emotions or attitudes within an individual toward the same person. An example would be a wife asking her husband, "Do you love me?" The husband responds with an angry tone of voice: "Of course, I love you!"

Boundaries refer to delineations between parts of a system or between systems. Boundaries must be clear and well defined for proper family functioning, yet they must be limiting and permeable at the same time.

Dysfunctional Family System refers to abnormal or impaired functioning. A dysfunctional family system has difficulties with communications, negotiation, and problem solving. There is usually reactivity, and acting out, instead of resolving family issues.

TWO TYPES OF DYSFUNCTION

Enmeshed family system: where the members are over -concerned and overly involved in each other‟s lives. Belonging to the family dominates all experiences, at the expense of separate sense of self-developing in members. The family over-emphasizes family belongingness. It is difficult for members to know what they want individually, and there is a rescuer mentality present.

Disengaged family system: refers to a family where members feel isolated. Communication is difficult and family protective functions are handicapped. Members

hardly respond to stress that one family member is experiencing. Only a high level of stress can reflect strongly enough to activate support from other family members. Forming relationships, both within the family system and outside, is difficult.

Functional Family System: is a system where members can develop a sense of self; there is a sense of "I-ness" in the members. Communication is clear and there is a pleasant atmosphere. Appropriate empathies are shown between family members, yet there is no urgency between members to rescue each other. There is mutual respect and equality. Each member has a voice and is able to have viewpoints, opinions and individual feelings. There is permission to move in and out of the system without friction. Boundaries are clear and well-defined, yet they are permeable so that the system can accept new ideas and growth as a system, and individual members can do the same.

MORE ABOUT FAMILY SYSTEMS

Family Systems theorists, mainly Bowen, maintained that there is a multigenerational transmission process taking place in family systems. Family rules, rituals, myths, beliefs and behaviors are transmitted from one generation to another, mostly unconsciously.

A) Open and closed systems

A system is a complex of component parts that are in mutual interaction. Rather than viewing each part as isolated and simply adding the parts to make up an entity, the family systems theory stresses the relationships between the parts. The various components can best be understood as functions of the total system. Systems differ in their degree of openness.

B) Closed System:

It admits no influence outside itself.
Is subject to disorder or disorganization.
Operates within its own impermeable boundaries, through which no information or energy can be transmitted.

C) Open System:

Tend to maintain a steady state or orderliness, or low level of disorganization, even though the latter may occur occasionally.
Exchange of materials, energies or information with the environment is possible because their boundaries are at least partially permeable.

CHAPTER 4: - ADULT CHILDREN OF DYSFUNCTIONAL FAMILIES

TRAITS OF A DYSFUNCTIONAL FAMILY

Dysfunctional families are created when dysfunctional individuals get married and are characterized by the defense mechanism denial.
A dysfunctional family keeps family secrets. They put up a front to the outside world and to each other.

In a typical dysfunctional family, may reflect:

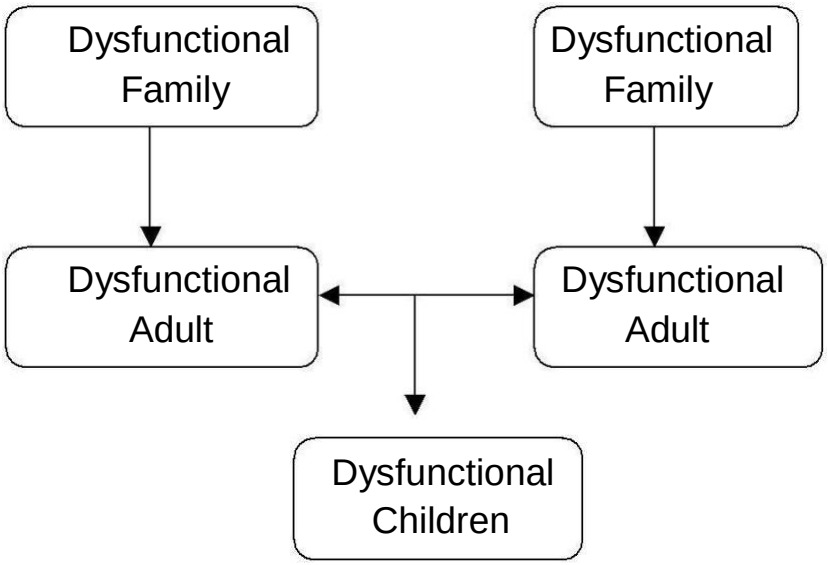
- Emotional, physical, or sexual abuse.
- Shame of parents, family, and self.
- Enmeshed relationships, lack of personal boundaries.
- Internalized feelings and free expression of feelings is discouraged.
- Manipulation to get needs met and to control others is common.
- A grim atmosphere within the family. Communication and openness is stifled.
- Very dependent relationships.
- Intimacy problems in relationships, they can"t trust people or get close to them.
- Chaotic family structure and unclear rules.
- Rigid attitudes.
- Reverence for the past, and for traditions.
- Frequent illnesses.
- Jealousy and suspicion.

TRAITS OF A HEALTHY FAMILY

Quite the contrary to a dysfunctional family, in a healthy family:

- Differences are encouraged.
- Self-expression and communication are present.
- Rules are explicit and clearly understood.
- There is respect for the individual with individual freedom.
- There is consistent value system.
- Open minded-ness encourages new traditions.
- Health is abundant.
- Independent growth fostered.
- Trust and love are present.
- Self-worth within members of the family is high.
- There are flexible boundaries.
- Individual personal growth is encouraged.

PROBLEMS ASSOCIATED WITH ADULTS FROM DYSFUNCTIONAL FAMILIES:



ROLES OF CHILDREN IN DYSFUNCTIONAL FAMILIES

Children develop four basic roles in a dysfunctional family.

Hero: This role is usually taken up the eldest child, who shows most of the following traits:

- Is a high achiever, always doing what is right.
- Type “A” behavior, workaholic.
- Usually marry dependent people who they can control and manipulate.
- a high need for approval, feel inadequate and that they can never do enough.
- Have
- Have a lot of anger, denial, and fear.
- Suffer from low self-esteem; and are very prone to suicide.
- Fill the self-worth role within the family through achievement.

Scapegoat: This is the problem child who acts out the underlying family problems. All the dysfunction of the family comes out in them. They show typical behavior as under:

- Hostility and defiance
- Getting into trouble with authority and the law.
- Feeling very inadequate with no self-esteem at all.

Feeling rejected and as adults, they often become substance abusers.

Lost Child: This child is the loner, the daydreamer whom nobody notices. They are high emotional sexual with most of the following issues:

- Abandonment, pain of being unloved and rejected.
- Does not feel “allowed to have feelings”.
- Are comfortable being by themselves and have a hard time making friends.
- Harbor feelings of loneliness and inadequacy. They lead unhappy lives and may die early.
- Have little awareness of their anger and are indecisive.
- As adults, they may become promiscuous.

Mascot: This is the clown of the family and will do anything for a laugh. Actually, they do things for attention and love and reflect the following traits:

- They are fragile, overanxious and have a high need to be protected.
- They also have a low self-esteem, and feel unimportant.
- They don‟t know why they are here.
- They usually marry later in life.
- They withhold a lot of secrets.
- As adults, they can‟t handle stress.
- They are dependent, so they marry the HERO.

In the above four roles, the parents‟ dysfunctions are being acted out in these different behaviors. These roles can overlap.

FAMILY SYSTEMS IN THERAPY:

In the family system approach, the family is viewed as a unit. The family is to be taken as one total organism with interaction and interdependence. In order to change the individual, you must look at and deal with the system.

A child who is the focus of a family system can‟t leave the system. He/she becomes enmeshed with the system. If the marriage and family are dysfunctional, there is a greater chance that the children will also be dysfunctional.
For example: A teenager getting into drugs or other kinds of trouble could be acting out the dysfunctions of the marriage. He/she becomes an addict to keep the family together. His/her unconscious purpose is to try to get the family help.

The identified patient is the individual expressing the symptom of the dysfunctional family. They are expressing a symptom of the system.

Stress Symptoms of the Family System:
Physical and Sexual Abuse

A child who witnesses violence is a victim of violence. A person‟s self-worth diminishes when they are beaten or severely physically punished. This can also lead to the condition known as “learned helplessness” where the person stops trying because no matter what they do, they can‟t win. They believe the situation is hopeless, so they give up. There is often a strong need to be perfect, which can lead to compulsive behaviors. Procrastination can come about because of the fear of failure or fear of success. Hostility and rage, or passive-aggressive behavior is common. There is often a lot of thinking of self or others as objects. Possessiveness, jealousy, and other controlling behaviors are typical.

In therapy, suppressed rage and anger can surface regarding the non-offending parent (why didn‟t they protect me?).

Sexually abused children often become compulsive adults. A child will often bond with the offender and then grow up to be addicts or child abusers. They may express their addictive behavior in many ways: alcohol, drugs, gambling, sex, work, exercise, or eating disorders.

Abused children grow up with problems dealing with trust. They can‟t trust people because their trust was broken by a family member. As an adult, they can‟t trust their feelings or thoughts. Either the person will avoid touch, or they will be promiscuous. They tend to have poor physical boundaries, free-floating anxieties, and shame-based, needy marriages.

Pseudo-Intimacy:
Emotional divorce is common in dysfunctional families. There are rigid boundaries within the system with the unspoken rule that the couple agrees not to disagree. They avoid areas that are potential disagreements and thereby block open communication and the possibility of growing closer.

They have a marriage that looks good to the world, but it is really based on “pseudo-intimacy” (the illusion of intimacy and health). It is really filled with pain, disappointment, frustration, bitterness and loneliness.

Co-Dependency:
Co-dependency is defined as taking care of another person‟s needs over your own; or a preoccupation with, or an over-dependence on, another person‟s needs or problems.

An example of co-dependency would be the spouse of an alcoholic working hard to cover up for, and protecting, the alcoholic (i.e. the wife calling to say that her husband is unable to work due to illness). It is behavior characterized by denial. Co-dependents are usually fixers or rescuers. They look for people to fix (alcoholics, addicts, etc.). The co-dependent is dependent on this as much as the alcoholic is on alcohol.

Co-dependents have trouble being spontaneous. They have problems being intimate. They often don‟t know what normal behavior is, because of poor modeling. They have an exaggerated need of approval from others. They tend to look for other people to care for. They need to control themselves and others.

Co-Dependents in Therapy:

The therapist should act like a feedback loop on how they are denying or avoiding their problem. Have them tell the story again, but this time without the smile. Look for their strategy for survival. Figure out which role they take on. Don‟t push them too hard; they will run if you push their façade of competence. Go slow. Don‟t go for cause too soon. Deal with symptoms until they bring up cause.

They will try to be the perfect client. They want your approval. They want to please you. They missed their childhood. Hence, they are adults on the outside, but children on the inside.

Give them love, acceptance, and nurturing. Build their trust, raise their self-esteem, and then you can deal with other issues. Give them relief from symptoms so they can go on to other issues. Get them to give up their family secret.

OTHER HIGH STRESS FAMILY SYSTEMS

Families that display compulsive behavior such as:

- Compulsive eating or dieting
- Work holism
- Gambling
- Exercising
- Spending
- Cleaning
- Families with constant arguing and tension.
- Families in which parents refuse to speak to each other for extended periods of time.
- Families with parents who have conflicting attitudes and values, or display contradictory behaviors that compete with their children‟s allegiance.
- with parents who are competitive with each other, or with their children.
- Families
- Families with a parent who cannot relate to others in the family, and thus, actively avoids them, while blaming them for this avoidance.

BY-PRODUCTS OF FAMILY SYSTEMS:

Inner Child:

Most of the times, when a child is born, it is in a state of completion and harmony. However, as the child grows and starts taking inputs from the environment, from parents etc, the mind starts developing a critical filter of judgment and starts operating on a binary system of duality.
In this process, whenever the child is exposed to any hostility either advertently or inadvertently, a small part of the child freezes in shock at that moment and stops developing
– leading to the creation of an “inner child”.

The development process is continued by the remaining personality which subsequently becomes the adult. The “inner child” created with the trauma and the memory of it - stays inside the adult and tends to get activated whenever this adult personality is faced with a trauma similar to that of the inner child.

Etheric Cords:

Whenever we develop an affiliation to any other person and etheric / energy cord automatically connects our etheric body to that of the person in question. This cord then becomes a transmitter for several emotions and energies between us and the person with whom we share the relationship.
Sometimes, due to any reason if the relationship goes sour and the cord is still in place it would leave us with a very distinctly uncomfortable feeling of not being able to get past the emotions and energies which the relationship was contributing.

All mothers have a natural etheric cord with their children.

Chapter 5: - CHILD HYPNOSIS

CHILD HYPNOSIS

For the purposes of this discussion, the definition of children will be eighteen years of age, and younger.

The first session should involve the parents, and if appropriate, siblings. The therapist should encourage the parents to attend the first session.

One of the first things the therapist must do is take the psychological and physiological history of the problem. This is especially important if the problem is enuresis (bed wetting). Also, be sure the history of diet and any medications.

MODALITIES

The following modalities may all be utilized in working with children.

1. **Hypnosis:** Use a very literal approach. Use “flowery” speech. Children like to fantasize. If, during the hypnosis session, the child wishes to go to the bathroom, let him go. If the child wishes to pull a cap over his face, let him. Basically, whatever the child wants to do, go with it.
2. **Play Therapy:** In Play Therapy, the therapist works with dolls to let the child tell them what the problem is. For example, use of “Mommy” and “Daddy” dolls will bring out conflict with parents.
3. **Mutual Story Telling:** In this technique, record the child telling a story. The therapist then tells a similar story with a better resolution into the tape recorder.

Points to keep in mind during working with children:-

- Changing Hats -The therapist has to symbolically change his /her thinking hat while working with children. Very little of adult approaches work with children. Unknowingly personal perception and judgments play a big role in a very subtle manner as the client is not in a position to defend or oppose. It is a good idea to keep one day a week for children work in which no adult work is done .That can prevent the adult context from spilling into children‟s work.

- Look for the children‟s world and associated problem.

- Fight/ Flight

One has to be sure that we do not trigger the flight mechanism in children as they can get manipulative and play on that later in times of stress. The therapy should be aimed at increasing the threshold of problem solving and mental tenacity. Therefore metaphysical work should be the last resort and done very subtly. It is essential to address the problem in a very light manner and raise the required skill set.

e.g – In case of a child who blanks during exams, work on the special energy coming to the body and mind during the exams.

Take the brief from parent and child separately.

- Usually children‟s problems lie in –Sibling rivalry, Academic underperformance – Scared or defiance towards authority. It is always better to work on the associated conditions through a CD recording that changes every week. Reinforce the missing skill set one at a time as the child is too much involved in the complication to let it go.
  Many a time it is also a comfort zone and an escape mechanism. In such cases a motivational build up is essential

- For children the word hypnosis gets changed to magic words etc. The countdown if from 10 to one with eyes fixed and the induction is shorter. It starts from the feet and goes to the head and from there the countdown can immediately terminate into reading of short direct to the point suggestions with visuals description as far as possible.

- For hyperactive children we ground them by channelizing the earth energy.

- Remember to ask the child about his positives before you ask or talk of the problem area. This turns the mind set to a more receptive state breaks down the defiance.

- Do not try to be very sweet to children as they would then loose objectivity and sharpness of the therapy will be hampered.

- Refrain from justifying the parents stand points. Many people before us have done it and it only creates an internalized resistance in the child.

- Respect the child‟s narration and look for concerns in his? Her narration rather than the parents.

HYPER ACTIVE CHILDREN

Causes of hyperactivity

- Heredity
- Chemical imbalance
- Emotional disturbance
- Electronic Addiction

We a therapist cannot do much in case of a genetically inherited ADHD, but the chances of it getting compounded are higher as any one of the parent may be also hyperactive .In such cases a regular practice of EFT to the parent for emotional issue clearance on a daily basis and a hypnosis to affirm that suggestion of daily reducing the stress levels works well.

Hypnosis for a calm and controlled mind is also very effective in case of chemical imbalance.
The kids need to go off electronics and the parents should be warned that they will face resistance but should be firm.

With children hypnosis for developing alternative interests should be done. Every time a new interest should be looked into and energy to be channelized towards sports through post hypnotic suggestions.

In case of emotional disturbance we should be focusing on academic, social and other areas of achievements and deal with them one by one.
With a hyperactive child, set the rules of the office immediately. Also, you as a therapist must be **very** literal.

Some rules to establish:

1. You will sit steady in any of the chairs in the office.
2. Let us take the five minutes challenge to sit steady.

CHILDREN WHO LIE AND STEAL

Reasons for lying:

1. Want to look better than they actually are.
2. Want to avoid punishment.
3. Negative Attention seeking
4. Compulsive –Secondary gain
5. Imitated

Reasons for stealing:

1. Abundance v/s scarcity
2. Peer pressure
3. Uncooperative and negative adults who are not in touch with the current needs of the children and use their childhood as a reference index.
4. Inconsistent parenting –where the child does not know his limits and has grown up believing that he can get whatever he wants by any means.
5. Desire for attention.

If the child is stealing, he is probably lying as well. Hypnosis for control over the temptations works well.

ENURESIS (BED WETTING)

Enuresis is considered to be a problem after the age of three. It is more common among males.

With enuresis it is important to find out if the parents were bed-wetters. If so, there may be a connection. Bed-wetters tend to be deeper sleepers and somnambulists. The child may be this way as well.

Part of the problem occurs when the somnambulist child dreams about bedwetting, and then does. The child does not wake up in this instance.

Have the child keep a diary of bedwetting (or any other problem). Get the story from the perspective of both parents and the child.

General Causes:

1. Indirect expression of anxiety.
2. Need for parental attention.
3. A desire to experience the pleasure of urinating.
4. Expression of unconscious hostility.
5. Deep sleep patterns.
6. Under-developed bladder tone.

Set up rules for the child. Some of these are:

1. The child cannot have liquids after dinner. If dinner is late, then no liquid with dinner.
2. The Child must urinate before going to bed.
3. They must urinate the first thing when they wake up in the morning.

In hypnosis, take the child through every detail of visualizing getting out of bed and going to the bathroom. Have the child feel the bed and see how warm and dry it feels. Have the child visualize being in bed and having to go to the bathroom. The instant they feel the need to urinate, their eyes pop open and they are “wide awake”. Have the child visualize swinging their legs over the bed, getting out of the bed, walking to the bathroom and urinating. When the child awakens in the morning, they have a nice warm feeling and a dry bed.

Possible problems:

1. The child might be afraid of the dark. Have parents use a night-light.
2. The child might not like the bathroom. Desensitize them to this.

A simple suggestion of getting up feeling dry and very happy with your self as you have finally grown up into a smart well controlled kid and all your little ittle problems have magically disappeared works excellently .

ANTI-SOCIAL BEHAVIOR

All anti social behaviors have their roots in LOW SELF ESTEEM .The therapist should look for reasons and triggers of low self esteem. Working only on the self esteem has known to produce wonderful results .Extreme pattern of parenting can often lead to defiance .Too little or too much care can

- The child seems to stay in the past. They have no regard for the present and therefore do not learn from their mistakes.
- The child is hostile and vindictive
- The child is extremely manipulative and untrustworthy.

The child demonstrates tremendous irresponsibility.

- There is a vast difference between their maturity and their intelligence level.
- The children are generally drawn towards friends and seek acceptance at any cost. This increases their risk of getting into negative company

The child lies and steals. Is very scared and puts up a defiance to cover the fear .But internally they are crushed by guilt and helplessness .We only need to touch that aspect.

You as a therapist must understand that the child is looser and need to reinforce the lost self image and self confidence .At times you may have to position yourself against the family during your conversations with the child .It is important to be subtle till the motivation level is built up as every word can be used for manipulations. Child -family is burnt out. Be patient! Changes must be made slowly.

There may be a lot of parental rejection and deprivation/Conversely too much perfectionisms and hyper attentive parenting. You will find that there is not a lot of family time spent together or too much focus on being controlled by family. There may be overindulgence or deprivation.

Help the child develop emotional security. Always give the child homework assignments (i.e. things to do during the week). Set up a work board.
Get a commitment from the parents to spend time with the child. This is interaction time, not time to punish the child. Quality time.

The therapist must also explore what is going on with the siblings. Find out who else is influencing the child (e.g. grandparents, aunts, uncles, school teachers, maid, etc.).
Establish a routine for the child at home. This teaches the child responsibility. If they miss an appointment, the child must call you … not the parent.

CHAPTER 6: - FIRST CONSULTATION

SUGGESTED SCHEDULE FOR FIRST CONSULTATION

15 minutes	Interview
	Find “Hook” – Known
10 minutes	Theory of Mind explanation
5 minutes	Suggestibility Test
50 minutes	Arm Raising
	Reactional Hypnosis
	Progressive Relaxation
	Suggestions
	Bring out temporarily
	Finger Spreading
	Suggestions
	Count out of hypnosis
10 minutes	Discussion with client.

FIRST CONSULTATION

The first consultation is a sales job. There are two types of sales:

- **Push**
- **Pull**

You cannot push someone to get better. However, you can pull them.
When you talk or make statements, you are pushing. When you question the client, you are pulling.

Rule: Don‟t ask questions that the client cannot answer. Only ask questions that they know the answers to. For example, “What would you like to accomplish?”

MINOR CLOSE

Ask minor reflexive questions. Ways to do this are as follows:

Tie Down: Make a statement, and end it with a question.
"This service is really valuable, isn't it?"
Remember, the therapist must be in control of the session, not the client.

Qualification: Finding out what is important to the client.
"How long have you been thinking about seeing a hypnotherapist?"

Win-Win Question: This is a question that gives the client an illusion of choice.
"Would you like to visualize or imagine a fully healed and healthy body?"

Open Ended Question: This is a question that cannot be answered with a one-word answer.
"When you are fully healed and healthy, how will it change your life?"

Establish a Need: Get the client to tell the therapist that the therapy is something that they want. *"What made you decide to see a hypnotherapist?"*

PAIN vs. PLEASURE

The client will want to remain in a pleasurable situation. Remember that the knowns in this case is staying the way they are (pleasure), while the unknowns are changing the way they are and going into therapy (pain).

If the therapist is in control, then the unknown (pain) is in control. If the client is in control, then the known (pleasure) is in control. If the therapist is asking questions, rather than giving answers, then the therapist is in control. To aid this, the therapist should always answer a question with a question.

GETTING PEOPLE IN THE MOOD FOR CHANGE

People come into therapy for subconscious, not conscious, reasons. They then rationalize their decision with the conscious mind.

People use emotions to make decisions. There are two types of emotions – Positive and Negative.

Positive emotions make people want to come into therapy, negative emotions do not.
As a therapist, do not bring up negative emotions.

Positive emotions are feeling better, success, etc. The therapist should create positive emotions that will help the client go forward.

OBJECTIONS

Remember that objections are defense mechanisms. When they object, throw it back with an answer, re-qualifying the therapy.

Integrity of the clinic.
"Is it me?" If so, then try to send the client to someone else.
Capable of service: The ability of the hypnotherapist to accomplish the goals. "Is it the money?" Talk about payment arrangements.

Chapter 7:- COUNSELING & INTERVIEWING

COMMON PITFALLS OF INTERVIEWING

Three bad styles of interviewing:

- Over sympathetic or empathetic
- Asking too many questions ("Just the facts")
- Need to make deep psychological interpretation right away.

A new therapist has an inherent idea of what his/her role is and will then cause one of the three problems listed above.

APPROACH-AVOIDANCE CATEGORIES

Therapist response categories were divided into two general classes: approach and avoidance reactions.

Approach reactions included verbal responses that were primarily designed to elicit from the client further expressions of hostile feelings, attitudes, and behavior. The following classes of responses were included in the approach category:

Approval

Therapist sanctions and expresses explicit agreement with the client"s hostile feelings or behavior.

C: I don't know but I got so mad when he came home.
T: Under the circumstances, how could you have felt otherwise?

Exploration

Therapist asks for further clarification, elaboration, and detailing of the client"s hostile feeling of behavior.

C: For some reason, I had a bad day; just couldn't fall asleep. I just felt aggravated. T: Aggravated? Can you tell me a little about that?

Instigation

Therapist shifts the discussion from a non-hostile topic to a hostile topic, or the therapist reintroduces the hostility expressed by the client earlier in the interview, or in previous interviews.

C: *I was pretty fatigued when I got home. I went to bed right off the bat but I just tossed and turned. When I woke up in the morning, I had this pain in the leg. I went to the doctor and he gave me a shot.*
T: Yes. Let's get back to that evening, and that irritated feeling you had.

Reflection
Therapist repeats or restates the client‟s hostile feelings.

C: *And when the kids don't listen to me, it rubs me the wrong way. I may even lose my temper.*
T: *You get mad.*

Labeling
Therapist names the client‟s feelings, attitudes, or behavior as hostile; points out patterns in the client‟s feelings or behavior; suggests relationships between present hostile feelings, behavior and past experiences.

C: *I've picked up the tabs on that girl all my life. Always paying, always costing money. I think I resented her a good deal.*
T: *Maybe some of your resentment for her is partly displaced from your mother, in that your mother shipped her to you to take care of. You were mad at your mother as well as at her.*

AVOIDANCE REACTIONS
Avoidance reactions included those verbal responses designed to inhibit, discourage, or divert the client‟s hostile expressions. The following classes of responses are included:

Disapproval
Therapist is critical of the client having felt or behaved in a hostile, aggressive manner.

C: *So I blew my top and hit her.*
T: Just for that you hit her?

Topical Transition
Therapist changes the discussion from a hostile topic to a non-hostile topic.

C: *My mother annoys me.*
T: How old is your mother?
Silence
Therapist makes no verbal response for 4 seconds or more after the client has expressed hostility.

C: *I just dislike it at home so much at times.*
T: (Silence)
C: *So, I just don't know what to do.*

Ignoring
Therapist responds to the content of the client‟s response, but ignores the hostile affect.

C: *I lose my temper over his tardiness.*
T: What are the results of his being tardy?

Mislabeling
Therapist labels feelings that are clearly hostile as non-hostile.

C: *When are you going to give me the results of those tests? I think I'm entitled to know.*
T: *You seem to be almost afraid to find out.*

CLINICAL INTERVIEW

The Clinical Interview is also called the "Psychiatric Interview", "Intake", and other names. Here it refers to the initial contact(s) with the client that is aimed at gathering information so that a diagnosis and appropriate Treatment Plan can be formulated. Except for the "Presenting Problem", most of the Clinical Interview is focused on historical information about the client.

IDENTIFYING DATA

Identifying data include name, age, date of birth (actual, not legal), marital status, education level, ethnic group, occupation, home address, phone numbers, email, etc.

PRESENTING PROBLEM

Use the client"s own words to the degree possible. Make sure the following is included: Symptoms reported by client
When the problem
began Course of the
problem
Precipitating event
Coping strategies of the
client ever had this
problem before?
Behavioral description of problem (Especially for child clients).

BACKGROUND INFORMATION (OR CLIENT HISTORY)

Family History:

- Ask about original family
- Who and how many? Father, mother, brothers, sisters?
- Family atmosphere – happy, tense, punitive, etc.
- How did you get along with family members?
- One closest to and why
- One farthest from and why
- Ask about current family
- Who and how many? Married, children, others living at home.
- Family atmosphere – happy, tense, etc.
- Dynamics – who is close to whom?

Developmental history of client:

- Pre-natal and birth information (if known to client)
- Pre-school years
- Developmental milestone information (late or early)
- What do you think those years were like for you?
- Any unusual trauma – physical, psychological,
- familial? Elementary school years (grades K-6)
- Grades client made
- Teachers especially liked or disliked
- Any unusual stresses or problem periods: at home, school, with friends?

Adolescence (Junior high school)

- Academic factors
- Friendship patterns – How many friends? Popular? Unpopular?
- Relationships with opposite sex
- Attitudes toward sex
- Any “rough times” psychologically?
- Any problems? Treatments received?
- Any unusual stresses – school, home, family, medical, etc.

High school years

- Cover the same factors as above for Junior High
- Individuation: Did you begin to break away from family?
- What effect did this have on family?

College years (if applicable)

- Academic factors
- Characterize these years
- Any unusual stresses – school, home, psychological, etc.?

Marriage or other life style

- What did you like about mate when dating?
- How do you feel about marriage (or lifestyle) currently?

Occupational history

- What job or jobs have you held?
- How do you feel about your current job?
- Any unusual stresses at work?

Financial history

- Any unusual stresses in this area?

SPECIFIC AREAS TO EXPLORE IN DEPTH

Medical and Psychiatric History: Get thorough history here – when, where, precipitating events, drugs etc.

Any other area needing in-depth exploration: Example, if client is seeking help for a marital problem, therapist would need to go into depth, taking history about dating, original reasons for marriage, course of events to date, and primary stresses in relationship.

SUMMARIZING & ENDING THE INTERVIEW

Ask: “Is there anything else at all that has occurred or is occurring in your life that might better help me to understand the problem?”

Goals of the client: “What would you like to work on, and what would be the goal?”

Summarize and formulate the problem, and make recommendations for treatment, further testing etc. Explain the next step carefully to the client.

CHAPTER 8: - EATING DISORDERS

NORMAL EATING

Normal eating is being able to eat when you are hungry and continue eating until you are satisfied. It is being able to choose food you like, eat it and truly get enough of it, and not just stop eating because you think you should. Normal eating is being able to use some moderate constraint on your food selection to get the right food, and not being so restrictive that you miss out on pleasurable foods.

Normal eating can also be:

- Giving yourself permission to eat sometimes because you are happy, sad, or bored, or just because it feels good.
- Three meals a day or it can be choosing to munch along.
- Leaving some cookies on the plate because you know you can have some again tomorrow, or it is eating more now because they taste so wonderful.
- Overeating at times, feeling stuffed and uncomfortable.
- Under-eating at times and wishing you had more.

Normal eating is, trusting your body to make up for your mistakes in eating.

Three Types of Eating Disorders

- Anorexia Nervosa
- Bulimia
- Compulsive Eating

ANOREXIA NERVOSA

Anorexia Nervosa is best described as a nervous loss of appetite in women. It is described in the Dictionary of Psychiatry as follows:

“The chief symptoms are deliberate restriction of food intake, weight loss, and in women, amenorrhea (absence of menstrual flow). Other characteristic features include faulty perception of body image, considerable increase in body activity, an intense fear of gaining weight, an accompanying mood change of depression, and unusual eating habits.

Other characteristics are:

- Intense fear of becoming obese, which does not diminish with weight loss.
- Disturbance of body image, e.g., claiming to “feel fat” even when emaciated.
- Weight loss of at least 25% of original body weight. If under 18 years of age, weight loss of original body weight plus projected weight gain expected from growth charts may be combined to make the 25%.

- Refusal to maintain body weight over a minimal normal weight for age and height.
- No known physical illness that would account for the weight loss.

Diet becomes a form of thinness. She thinks it gives her all she lacks. In actuality, she is emotionally depleted, and has no strong emotional foundation.

An anorexic will develop impulsive/compulsive behavior (if this is not already part of her pattern). Diet becomes a forum to act out her impulsive/compulsive acts.

Pre-Anorexic State

The client is overly concerned with intelligence and appearance. She has a strong tendency to overcompensate. She is very impulsive at this time, acting before thinking things through.

As the disorder progresses, her activity level decreases. She has a tendency toward "splitting", seeing decisions in terms of polarity, everything is all black and white. Because of this, she is unable to make a decision. She seems very immature, and can"t handle things in a mature way. She wants to remain a child, and has no desire to grow up. All her actions are those of someone not fully developed.

Fears: The fear of criticism is a major concern. The client will think she is fat when she is not.

Passive-Aggressive behavior: She is passive in dealing with people. The disorder is a tool to deal with aggression.

The Anorexic State

Here, the client will experience lots of anxiety. Diet represents an escape from the threat of obesity, no matter how thin the client is. Fat to her implies failure. The client has a tremendous fear of rejection, mostly by her father, or the secondary caretaker.

The anorexic will experience hunger. The client will experience a lot of phobic thoughts: fear of weight gain etc. She will then deal with her fears in overcoming hunger.

25% get into the binge-purge cycle that is characteristic of Bulimia.

She will continue her obsessive thought process, and lose her emotional connection to anything but food. She will set goals for herself: lowest weight, greatest weight loss, etc.

She will exhibit obsessive-compulsive acts: spending a lot of time cutting her food, lacing and rearranging everything on her plate so it is “just right”, etc.

- Delusional thinking: The client will lose all sense of proportion, especially concerning weight or food.
- Denial: The anorexic will go through tremendous denial, to the point where she will deny that she is ill. She will also evidence as a form of denial.
- Food habits: The anorexic may go as low as „a 300 calorie per day‟ diet, and also completely cut out entire food groups. Hypoglycemia is also very evident in anorexics.

Some anorexics will become very avid exercisers, which in their state will eat up the muscle and make them thinner. They will also abuse diuretics, enemas, laxatives, thyroid pills, etc.

Physical Problems

- Loss of menstrual period.
- Scalp hair becomes very thin and brittle. You can see right through to the scalp.
- Severe constipation - the anorexic will experience a lot of constipation. This might be because of excess use of laxatives or bad eating patterns.
- Lower blood pressure.
- Lower blood pressure.
- Lanugo - The body temperature may drop to as low as 95 degrees. The anorexic will begin to grow a fine growth of hair all over her body in the body‟s attempt to retain body heat (like a regression to primitive man).
- Lower pulse rate - the pulse rate may drop to as low as 39-60 beats/minute.
- Some dental decay - this is more present in cases of bulimia, but will also be evident in anorexics who binge-purge.

Causes of Anorexia Nervosa

The chances are that the family is dysfunctional, making the problem a systems problem.

Part of the problem is caused by the Emotional father. She wants acceptance and approval from her father or secondary care taker, but he is unable to give it to her. She creates her own little world, and an intense desire to be perfect, to get the acceptance and approval she desires.

Once the dieting starts, she will withdraw from the family and activities. This doesn‟t work because of the weaknesses of both the Identified Patient and the family. She will therefore push herself past limits, and the rest of the family then begins to notice.

They now “take action” by trying to feed her. The anorexic thinks that the whole world is out to fatten her up. The anorexic then feels out that she cannot trust anyone.

This will solicit concern from the family, and the family rallies around her because she is sick. At this stage, the therapist deals with the problem on a systems level.

Control, becomes a big issue. Growing up, puts her out of control since, this involves going through puberty, and moving out on her own. These things create a phobic reaction to her impending sexuality. It means an end to her childhood, and separation from parents. She has no identity of herself. The anorexic solution to the problem is to not grow up. She does not want the rounded look of an adult woman.

The anorexic is seen, by the family, as an adolescent out of control.

She feels a lot of demands, and picks up a lot of wrong inferences from her Emotional father. Since, she herself perceives lack of nurturing she feels it is her job to nurture the family. The anorexic will overcompensate by nurturing everyone from an early age.

Rituals in Obsessive-Compulsive Behavior: These are designed as a safety net. If she is perfect, then everything is all right.

Treatment Approaches

The decision to enter therapy must be the anorexic‟s. If not, the therapy will last only a few sessions. If the client has come in as a last resort (they have been elsewhere before), don‟t talk about the disorder, talk about what they are feeling.

Anorexia requires rapid therapy. First, desensitize the anorexic to weight gain. Forget the numbers of the weight; just go for the gain. Don‟t get caught up in arguing with the client over goal weight. If you do, you could spend many sessions arguing over five pounds.

Build a good, solid, emotional foundation, helping the client to grow emotionally. This aids the client to grow in confidence and self-esteem. Slowly, take away their defense mechanism. Get rid of the inference that she is perfect. Work with the family, especially in how the Identified Patient has worked in the family system. Remove her chance to nurture. Encourage the family to give UNCONDITIONAL love (to others as well as to her).

The family must refrain from nagging her regarding her weight loss. Teach them to create a loving environment. Remove the nurturing gratification that an anorexic gets.

MISTRUST is a core problem.

98% of all anorexics are Emotional Sexuals. Use this, since the relationship with the Emotional father is at the base of the whole problem. Expose the love-hate relationship with her mother.

Desensitize her, and block her to her dysfunctional family. Establish lots of feelings of control. Work first with issues with the father and then the mother.

BULIMIA

Definition from Psychiatric Dictionary: "Insatiable hunger. An eating disorder consisting of a pattern of eating binges during which a large amount of food is ingested in a short period of time. Each binge is typically followed by a depression in mood and disparaging self-criticism, and, in some cases, an imposition of a rigid diet."

Some typical characteristics are recurrent episodes of binge eating (rapid consumption of a large amount of food in a short time), along with at least three of the following: Consumption of high caloric, easily ingested food during a binge.

- Inconspicuous eating during a binge.
- Termination of such episodes of eating by abdominal pain, sleep, social interruption, or self-induced vomiting.
- Repeated attempts to lose weight by severely restrictive diets, self-induced vomiting, or use of cathartics or diuretics.
- Frequent weight fluctuations of greater than ten pounds due to alternating binges and fasts.
- Awareness that eating pattern is abnormal and fear of not being able to stop eating voluntarily.
- Depressed mood and thoughts following eating binges.
- The bulimic episodes are not due to Anorexia Nervosa or any other physical disorder.

Bulimia is most associated with the binge/purge cycle. It means "Insatiable Appetite". Purge, as noted above, is not necessarily vomiting. According to Webster"s Dictionary, purge means "Getting rid of guilt."

THE BINGE

During a binge, the number of calories a bulimic may take in is approximately 10,000 calories. The binge food does not have to be only sweets. The bulimic will binge on carbohydrates and the binge could last as much as four to five hours.

Bulimia is an addictive disorder, just like alcohol and substance abuse.

Physical Symptoms

- The stomach becomes extended.
- The eyes blacken. This is due to blood vessels under the eyes breaking. This can never be reversed, even if the client is cured.
- Swollen jaws.
- The teeth are discolored and decaying.

Health Risks

Some bulimics will vomit if they have as little as a glass of water to drink. The vomiting becomes involuntary. Once this happens, they may even vomit in their sleep. Make sure the client does not eat before they come in for a session. The client could rupture the esophageal wall, which would result in instant death.

Bulimics substitute vomiting as a pleasure (i.e. substituting sex, release from overload, lack of love, physical activity, etc.).

The biggest problem is to build a strong emotional base. Also, watch for lower potassium and chloride levels as a result of the purge.

Maintain a good rapport with a medical doctor, so they can handle any medical emergencies.

Similarities and differences between anorexics and bulimics:

The bulimic is more mature socially. Usually, she is sexually very active. An anorexic could care less about sex.

Most anorexics or bulimics are hypoglycemic. It is very important for them to be on a proper diet.

Another trait of anorexics and bulimics is that they constantly lie.
Most bulimics become substance abusers to fill voids.

Most bulimia occurs between 14 and 45. The average age is 23. Bulimics may include 10-15% men. Men who have this disorder include jockeys and dancers.

Most anorexia occurs in girls between 12 and 25 years of age. 14 is the average age. On an average 21% of all anorexics die.

Many anorexics and bulimics are victims of child abuse.

HYPNOTHERAPEUTIC APPROACH:

Bulimia: Put in a stop mechanism to the binge or purge. Use circle therapy, bring up the feelings, and tell them, “Stop it, pass it, go deeper.”

Use dream therapy to get to the cause. Childhood memories are sometimes not pleasurable, or not there at all.

CHAPTER 9: - HABIT CONTROL

OVERVIEW

Major habit controls that will be dealt with as a hypnotherapist:

- Nail biting
- Tardiness
- Weight
- Study Habits
- Smoking (dealt with in Level 5)

NAIL BITING

There are two main reasons for nail biting:

- Nervous habit.
- Response to anxiety.

If it is a nervous habit, when it is brought to their attention, they will stop. If it is a response to anxiety, the client will get upset when it is brought to their attention. To find out which it is, ask the client how they react.

If it is a nervous habit, treat it symptomatically. With anxiety, desensitize it. If the client can remember the cause, have them vent it out.

Way to desensitize:
Have the client imagine feeling calm. If they bring their hand up to bite the nails, the muscles tense up. It feels good to put the arm and hand down.

TARDINESS

Tardiness is either avoidance or a defense mechanism. It is used to avoid discomfort or what is perceived as pain. There is a difference in treatment with Physical and Emotional Sexuals.

Once you find out what the problem is, attack it directly.

With tardiness, the Physical wishes to avoid anxiety of being on time, because he/she doesn‟t like waiting around. Work on relieving the anxiety of arriving early.

The Emotional likes to be on time, or even early, because they don‟t like being late. If an Emotional is always tardy, they are generally obsessive-compulsive. Give them time cushions, and make it a habit.

WEIGHT CONTROL

Weight control is the most difficult of all kinds of therapy. The reason for this is that 95% of all weight problems are emotional problems. Therefore, the therapist must always go for cause.

It is a tough, challenging therapy. The best success is where eating is just a habit. If the weight is emotional, it is a very long-term therapy.

Some causes for weight problems:
Weight gain may be a defense mechanism against relationships. This includes past, present, and future potential relationships.

- Some people eat out of boredom.
- Guilt is often the cause of people gaining weight. Eating may be a consolation to the guilt.
- Eating habits, either gaining or losing weight, could be the result of the end of a relationship. When relationships end, Physicals tend to lose weight whereas Emotionals tend to gain weight. If the opposite occurs, look for cause (eating replaces the lost relationship).

Resistance
Occasionally a client may be very resistant to weight loss. Some people‟s appetites are satisfied by sight and smell, rather than by taste. Instead of telling them to avoid food, or avoid looking at food, have them bring in pictures of food from magazines (these should be very appetizing). Take the pictures and put the client in hypnosis.

In hypnosis, tell the client, “Imagine a wide belt around your stomach area. As your eyes take in the food, the belt is tightened a notch, and you feel full.”

Repeat this suggestion several times, using both sight and smell sensations. Suggest that the sight and smell of food is so satisfying that when you see or smell food, it will increase your feeling of fullness. Then bring the client out of hypnosis, and show them the pictures.

STUDY HABITS

Main reasons for poor study habits:

- An anxiety state, experiencing avoidance and procrastination.
- Poor comprehension and bad reading habits.
- Inability to organize material effectively.

The key is to find out what the client is avoiding. If they are studying a boring subject, create interest in the subject. With an Emotional, tell them that if you can‟t study, you‟ll flunk.

Find out if the client is eye or ear minded (question from suggestibility test). If they are ear-minded, have them read out loud, or into a tape recorder.

Avoidance is difficult to work with. Procrastination is a defense mechanism. You must meet it head on and work on it symptomatically.

Another way to work with this is to have the client imagine himself studying and feeling comfortable. Then have him imagine himself not studying, taking a break, and feeling uncomfortable.

EXAMINATION ANXIETY

Use desensitization to deal with this. Desensitize the client to the anxiety. Teach them self-hypnosis to reinforce this at home.

Break the condition of the exam room creating anxiety. Have the client feeling calm and peaceful in the exam room. Have him/her recalling all the material needed for the exam easily, with no discomfort.

Create a visualization of the exam being over, and the client leaving the building, feeling really good about successfully answering all the questions. *This works very effectively with an Emotional.*

CHAPTER 10: - SEXUAL DYSFUNCTION

OVERVIEW

Sexual response is a complex series of reflexes that work well only if a person is in a calm state and is left alone, i.e., not through the conscious mind.

When the sexual response screws up, it has a ripple effect upon the rest of the life of the person(s) involved. For example, impotence will affect a man‟s entire life – to the point where some men will commit suicide.

Help the client to keep a perspective on sex. It is a ten-second function that has become so important in today‟s society, mainly due to media hype.

ANXIETY

Anxiety is the big enemy of good sexual response. Outside influences that may affect good sexual response are alcohol, cigarettes, and drugs, since all three tend to restrict the blood flow.

Anxiety will inhibit sexual response. If a man or a woman becomes anxious about having an erection or an orgasm, it probably won‟t happen.

Sources of Anxiety:

- Avoidance or failure to engage in sexual behavior that is exciting and stimulating to both.
- Fear of failure.
- Over-concern about pleasing partner, i.e. fear of rejection.
- Tendency to erect perceptual and intellectual defenses against erotic pleasure.
- Failure to communicate openly and without guilt; and defensiveness about genuine feelings, wishes and responses.

PLEASURING

For most sexual dysfunctions, a great deal of non-demand type of touching between the partners (using the fingertips, etc.) can be very useful. This is called “pleasuring”.

If a couple comes for therapy, in addition to pleasuring, the couple is not allowed to have sex until the therapist allows them to. In a few cases, this by itself is enough to relieve the problem.

The reason for this is that telling someone with a sexual dysfunction that they do not have to have sex reduces the performance anxiety.

For this to be effective, the scene needs to be set. The children need to be put away somewhere (preferably out of the house), the telephones should be turned off, there

may be music playing in the background, and there may be fragrances and lit candles.

Again, with pleasuring, the couple does not have sex until the therapist gives them permission. During pleasuring, verbal communication is very important. It is important for each partner to tell the other what feels good to them and what does not. Communication is the key to any correction of sexual dysfunction.

PREMATURE EJACULATION

This is a situation where the male cannot control himself and ejaculates at or before penetration. A man who has this problem is unable to control it.

This, generally, is a learned response, going back to the first sexual experience. In almost all cases, the problem is an emotional one, not a physical one. Examples of this may be:

- On the girl‟s sofa with the parents due home at any time
- In the back seat of a car at the drive-in movie with the attendant due to check at any time
- Going to the hooker for the first experience, where the hooker wants it done quickly so she can move on to the next customer.

If the first response was a rapid ejaculation, it creates an anxiety situation. Since premature ejaculation is a learned experience, control over the ejaculatory reflex is not learned.

Ejaculation is a reflex action, just like urination is. Therefore, more control over it is possible. In the past, doctors tried using distraction as a cure (e.g., counting backwards from 100, thinking the alphabets backward, etc.). However, this is not a cure. Neither are creams, gels, etc.

To work with this problem, have the partner use the “squeeze technique” that high-class call girls use with their clients. This technique will clamp off the ejaculation, while bringing the man to a state of arousal. If this is done enough times, the man will learn to control the ejaculation. Generally, the man must tell his partner when to squeeze, or it won‟t work.

A variation of this is the stop-start technique, which may not be as reliable as the squeeze technique. First, have them practice pleasuring. When the couple progresses to genital pleasuring, and the man feels the ejaculation is near, he tells his partner, who will stop him by the squeeze. Then the man is aroused again by genital pleasuring. The man ultimately becomes aware of the sensation of being able to stop, and gains control.

Another technique is to have the partner mount the man and just sit there, so that the man can feel the vaginal containment without ejaculation.

The success rate for premature ejaculation in normal people is close to 100%. However, there is also communication between the partners and the therapist to work out the problems of hostility that have come up during the curing of the dysfunction. Help the couple realize that this sacrifice will benefit both partners.

In working with a couple, it may be a good idea to hypnotize the couple together, since the problem is one that affects the entire marriage, and thus both must be worked on together.

IMPOTENCE

A side effect of the anxiety and the tension that the dysfunction creates is impotence.
In this case, there may be both psychological and physiological factors contributing to the condition. For this reason, it is very important for the client to have a physical examination.

The results of this may be anticipated by finding out if the client has erections upon awakening in the morning. If he does, the problem is probably not a physiological one. However, there are so many physical possibilities that an examination by a medical doctor is necessary.

After a physical examination, if it is determined that this is an emotional problem, get both partners into therapy, since it is a joint problem. The treatment must be done together. The wife must be shown that she is not the cause of the problem, but she will be part of the cure.

First, have the couple pleasure each other, and do not give themselves permission to have sex. This removes the demand for sexual performance, and the resultant anxiety. It will also open up communication, especially during the therapy hour. Clear the air of all the emotional stuff going on that has not been discussed between them.

During pleasuring, also open the lines of communication. Have them find out what their partner enjoys. This will renew their ability to give each other pleasure, while making each partner feel good about each other.

The man may now get an erection, which will give him confidence and security, which is the goal of the therapy. Once that goal is reached, the impotence problem will generally be cured.

FEMALE DYSFUNCTION

Typically, the woman who does not have orgasms during intercourse is the most common type of client. For the woman who does not have a partner, have her either get a vibrator or use self-stimulation.

In the case of a woman who has a partner, use pleasuring, starting with no vaginal contact. When they reach genital stimulation, have the woman show the man what “turns her on” and stimulates her. She may have to do it herself in order to show him.

In order to have orgasm during intercourse, clitoral stimulation may be necessary, either before or during the intercourse. The key is to have contact with the clitoris during the intercourse.

During orgasm, contraction of the PC muscle is what gives the woman the pleasure. There is an exercise to strengthen the muscle while urinating. In essence, the woman would sit on the toilet while urinating, and then stop the urination. Women can practice this exercise up to 60 times/day (on the phone, driving the car, etc.). The exercise was originally designed for women with urinary problems. However, women found that using this aided them in having an orgasm, and increased their sexual pleasure.

CHAPTER 11: - CRISIS INTERVENTION

OVERVIEW

It is important to read the signs of the client, and get the feelings of the mood of the client. Helplessness and hopelessness is what makes the crisis a crisis. Generally, the crisis is a long build-up of other things going on in the client‟s life. Don‟t minimize or maximize the crisis.

Give the client *hope*.

It is important to let the clients talk without interrupting them. This gives them a chance to vent their feelings that they might have been suppressing. The therapist must keep himself in an objective, non-judgmental and caring posture.

It is important to listen and draw the client out. Have them change their posture: sit, stand, etc. Don‟t do for the client what they can do for themselves. Break down the problem into solvable chunks, rather than attack the whole thing at once.

WHAT IS A CRISIS?

A crisis is a life event associated with a significant deterioration of the normal ability of the individual to resolve his/her problems.

In the crisis, there is a high degree of emotional stress. When the emotional part of the brain functions on overload, the analytical part cannot work. This may cause judgment errors, which may cause negative consequences, thus lowering the client‟s self-esteem. This aids hyper-suggestibility, negative input and emotions.

Physiological impairment may also occur. Many studies have shown what happens to the body under stress.

EXPLOSIVE PERSONALITY DISORDER

There are two types of explosive disorders: Intermittent and Isolated.

Intermittent Explosive Personality Disorder is defined by:

- Several discrete episodes of loss of control of aggressive impulses resulting in serious assault or destruction of property.
- Behavior that is grossly out of proportion to any precipitating psycho-social stressor.
- Absence of signs of generalized impulsivity or aggressiveness between episodes.
- Not due to Schizophrenia, Antisocial Personality Disorder, or Conduct Disorder.

Isolated Explosive Personality Disorder is defined by:

A single, discrete episode in which failure to resist an impulse led to a single, violent, externally directed act that had a catastrophic impact on others.
The degree of aggressiveness expressed during the episode was grossly out of proportion to any precipitating psychosocial stressor.
Before the episode, there were no signs of generalized impulsivity or aggressiveness. Not due to Schizophrenia, Antisocial Personality Disorder, or Conduct Disorder.

The key point to remember is that the aggression happened under stress. The aggressive act may have been an overreaction, but it did occur under stress.

TREATMENT

The therapist as a neutral, non-judgmental listener understands that the client is carrying some emotions, thoughts and associated energies inside.

A technique known as “Power Blasting” can be used for effective results.

- First, get the client to talk about the problem.
- Ask if the problem is inside you or outside you.
- Then, have them turn it into a symbol.
- Next, have the client go inside their body and find out where in the body it bothers them. (Body Syndromes).
- Ask if the client can get rid of it without hurting himself. If the client cannot, then have them only get rid of a part of it, not the whole thing.
- Then have the client go into hypnosis, and move the symbol outside their body near the point in the body where it bothered them.
- Have the client plug their feet into a Universal Power Source (White Light), and see the energy around the symbol until it is totally.
- Then blast everything out with a deep breath.

PRIVATE VIOLENCE

1. Wife Beating
2. Child Abuse
3. Incest
4. Rape

All of the above have a common thread. All the acts are both violent and private.

Also, people have a hard time talking about the fact that they have been beaten.

WIFE BEATING

Wife beating is more prevalent than previously thought. It is common in all racial and economic groups.

There is a cycle of violence that is commonly followed in these cases:

Tension Building Stage:

- Persons are unable to communicate, giving the “Silent Treatment”.
- Losing Control Stage:
- At the end of this stage, there is some act of violence.
- Loving & Remorse Stage:
- The man is generally genuinely repentant.
- Setting Stage for the New Cycle:
- Some problem develops, which will trigger the cycle to happen all over again, but this time.
- He will lose control even more (going into Fight-Flight).
- The frequency builds up, especially if the wife accepts part of the blame.

Another common thread is that both husband and wife have low self-esteem.

Common Traits of the Husband:

- Jealous of partner
- Control and Isolation of partne
- Jekyll and Hyde personality
- Explosive Temper
- Legal Problems
- Birth of a Child

Common Traits of the Abused:

- Low self-esteem
- Acceptance of Male Dominance
- Equates Dominance to Masculinity
- Accepts Partner‟s View of Reality
- Bases her Self-worth on him
- Financial Dependency
- Fear
- Strong Family and Religious Values

“A man beats his wife because he can”: this is a common statement regarding wife beating. The answer to this is, “A man doesn‟t beat his wife because he can‟t.”

If the husband is on alcohol or drugs, it is probably best to advise the client to leave.

CHILD ABUSE

Signs of Physical Abuse

Bruises:

- On any infant, especially facial bruises.
- On the posterior side of the child"s body (largest percentage of all abuse shows here).
- With an unusual pattern that would show an instrument or bite mark.
- Clustered bruises, indicating repeated contact.
- In various stages of healing.
- On multiple surfaces of the body, and both sides of the face.
- Inconsistent with the story given, or unexplainable.
- Whipping or wrap-around bruises, loop marks, doubled-over cord.

Timing or dating:

Within a few hours	Red
6-12 hrs	Blue
12-24 hrs	Black-Purple
4-6 days	Green Tint, Dark
5-10 days	Pale Green to Yellow

Burns:

- Immersion burns (stocking burns, burns on arms and legs, donut shape on buttocks), indicate dunking in hot liquid.
- Cigarette burns (usually multiple, and found on palms or soles).
- Dry burns or brands (indicating a hot instrument applied to the skin).
- Zebra burns (holding under a hot water faucet).

Other (Fractures, Head Injuries, Pummeling)

- Any fracture in a toddler or infant is suspect. Long bone spiral fractures from twisting are almost always due to inflicted trauma. “Chip” fractures at the end of a long bone. Rib fractures, especially in the back.
- Absence of hair and/or hemorrhaging beneath the scalp due to hair pulling. Sub-dural hematomas (hemorrhaging beneath the outer covering of the brain due to striking, hitting, or pummeling). Retinal hemorrhages or detachments (due to shaking). Whiplash.
- Shaken Infant Syndrome (intra-cranial hemorrhage), no external signs but causes serious damage associated with learning disabilities in later years. This is one of the most deadly forms of child abuse.

Signs of Emotional Abuse

Excessive verbal assaults, unpredictable responses, continual negative moods, constant family discord and double messages are all signs of emotional abuse.

Suspect if the Child is:

- Withdrawn, depressed, apathetic.
- “Acts out”, and is considered a real behavior problem.
- Overly rigid in conforming to instruction from adult authority.
- Shows other signs of emotional turmoil such as repetitive rhythmic movements
- Inordinate attention to details
- No verbal or physical communication with others.
- Making unwitting comments, such as “Daddy always tells me I‟m bad”.
- Referred to by the parent as IT, instead of him or her.

Traits of Child Abuse Caretakers:

- Many have experienced abuse themselves, or are also involved in domestic violence.
- Many reverse roles with their children. They demand love and care from their offspring, but have difficulty returning emotional feelings.
- Crippled by their own experiences of abuse, they often do not understand how to get their own needs satisfied, much less have the skills needed to meet the basic needs of their child.
- Mothers who have little contact with their infant immediately after birth.
- Premature birth, or born with an illness, or where hospitalization of the mother or child disturbs bonding.
- Caretakers of a child with physical defects.
- Many have a tendency toward social and emotional isolation, and often they do not know how to find or connect with support systems.
- Many are in a vicious cycle of low self-esteem and negative self-worth.

One or more of the following factors could trigger abuse:

- Emotional stress: Marital, employment, significant loss.
- Strong belief in corporal punishment.
- Lack of constructive outlets for anger and aggression.
- Any act of the child that triggers the contempt or resentment of the caretaker.

INCEST

Often times, the therapist will find family problems at the root of the incest. The victim of the incest will begin to assume the mother role in the family. She becomes distant to her peer group, and withdraws, and sometimes runs away. Sometimes, she will get involved in substance abuse and her mother will look the other way.

Characteristics of Incest Victims:

- Low self esteem
- Poor self image
- Role reversal – plays wife or mother instead of child
- Appearance of maturity but has feelings of immaturity
- Has had poor mothering
- Denies own needs
- Feels objectified
- Confusion of self as sex object, love object
- Depression
- Suicidal thoughts or attempts
- Depersonalization, multiple personalities
- Denial
- Repression
- Guilt, anger
- Anxiety, fearfulness
- Feelings of rejection
- Feelings of sham
- Withdrawn, apathetic
- Anti-social delinquent behavior
- Truancy, drop-out
- Problems in school adjustment
- Runaway
- Social isolation
- Drug abuse
- Self-mutilation
- Somatic complaints
- Feels anger toward father – projects to all men
- Confusion regarding sex
- Poor body image
- Fearfulness of world outside the family
- Holds other women in contempt
- The perpetrator is often known by the child.

- Children are often conditioned to comply with authority.
- Children are in a subordinate position and fearful of threats.
- Children are curious and susceptible to bribes and rewards.
- Children are naïve about social norms and values.
- Children are sensual beings who respond willingly to intimate and gentle contact, which they may associate with feelings of being loved, cared for, and cherished.

Females are victimized in the ratio of ten females to one male. It rarely exists as a single event. It may begin innocently enough, from touching and caressing to fondling. Most children develop, at a young age, a sense of it being wrong. Younger sisters are also often victimized. The overwhelming majority of perpetrators are men.

Violence is uncommon. For some, it may be seen as the first instance of special attention from a father figure. As with some battered children, negative attention is often better than none at all.

Most perpetrators don"t fit the nightmare child molester stereotype. Many have poor self-concepts and dysfunctional relationships, but are not considered “sick” in the traditional sense.

RAPE

The therapist must get across the idea that rape is a violent crime, not a sexual one. This may involve handling problems between members of a relationship.

The therapist may need to desensitize the husband or boyfriend. Some form of sex therapy is generally indicated. A good approach is to a temporary ban on sex.

The therapist needs to help the victims understand that they are victims. Get the client to write a letter or letters as follows:

- The strong self writes a letter to the weak self, and vice versa.
- The victim writes a letter to the perpetrator.
- The victim writes a letter to the husband or boyfriend.

In all these letters, get the client to let out the anger and allow them to accept the victim role. Also, tell her that the letters do not have to be mailed unless she wants to mail them.

The rapist usually rapes his victims for power, and not for sex. With most perpetrators, sex is usually available.

Treatment

Remember, the victim does not have to re-experience the violence of the act. Using desensitization, have the client watch on a television screen the events up to five minutes before the rape. Using ideomotor response to gage her feelings, review the scene on a television screen moving closer to the actual event, never letting her experience the full force of her feelings.

Also, the client can deal with this from a *Resource State.* This is a state of strength. Once the client can see the event while maintaining the *Resource State,* then they would walk over to the pained person on the television screen and comfort them.

CHAPTER 12: - DEFENSE MECHANISMS

SIGMUND FREUD

Sigmund Freud (1856-1939) believed that three central forces shape or "constitute" the personality: instinctual needs, rational thinking, and moral standards. All these forces, according to Freud, operate at the unconscious level, unavailable to immediate, cognizant awareness. He believed these to be dynamic, or interactive, components whose vying for expression shapes the person‟s behavior, feelings, and thoughts. Freud constructed a final, *structural* model of the mind, calling these forces *id*, *ego*, and *superego* therein.

Freud used the term *id* to denote the instinctual needs, drives, and impulses. Operating on the *pleasure principle*, Freud believed by people are primarily motivated by the id. This, as characterized by Freud, is *primary process thinking*. Freud created the concept of *libido* to represent the sexual energy that fuels not only the id but other forces of the personality as well. Freud also believed that the id has a death instinct – *Thanatos* – that creates the life-death conflict within the individual.

The *ego*, according to Freud, develops out of id. Like the id, the ego unconsciously seeks gratification, but it does so in accordance with the *reality principle*, thereby gaining control over the expression of id‟s impulses. The ego, employing reason and deliberation, guides us to recognize when we can and cannot express those impulses without negative consequences. The ego assesses new situations, reflects on past experiences, anticipates consequences, and plans how best to obtain gratification – all of which constitute Freud‟s concept of *secondary process* thinking.

The *superego* develops out of the ego, just as the ego grows out of the id. As we learn from our parents that many of our id impulses are unacceptable, we unconsciously incorporate our parents‟ values. We identify with our parents, and judge ourselves by their standards. When we uphold their values, we feel good; when we don‟t, we feel bad. The superego is our conscious, and embodies an *ego ideal,* that is a composite image of the values acquired of what we should be.

The classic Freudian model views the contents of the unconscious (unconscious memories, strivings, and wishes) as so-called derivatives of the instinctual sexual and aggressive drives, having nothing to do with the outside world. In contemporary psychoanalytical thinking, the human being‟s subjective world evolves from the mind/body system‟s interaction with the environment. That is to say, the contents of the individual‟s unconscious derive from a complex interplay of genetics, biochemistry, and his/her surroundings; therefore Freud‟s notion that the id (the "cellar" of unconscious fantasies) is cut off from the external world is no longer tenable. With this in mind, we

can appreciate Freud‟s discovery of the unconscious system and the ego‟s defensive strategies with greater breadth and depth.

DEFENSE MECHANISMS

Although the individual is unaware of them, defense mechanisms serve to protect the ego from anxiety, enable the personality to cope with the pressures of reality, while continuing to repress id‟s unacceptable desires. Since the individual is unaware of the ego defending itself, the analyst can only infer from the behaviors what the defenses are. If these protective defense mechanisms break down before the ego is strong enough to deal with the repressed thoughts, anxiety becomes unbearable, and this may be what causes a person to opt for the ultimate escape – psychosis. Decompensation is a progressive loss of normal functioning in favor of psychotic behavior.

All of us use coping strategies of many types, including defense mechanisms. At one time or another we all have unhappy experiences, are faced with problematic situations, and have obstacles to overcome. We all experience disappointments and failures at various times in our lives. We all need to feel good about ourselves, but our self-esteem is always at risk, making us vulnerable to negative experiences. When circumstances threaten self-esteem, we tend to become defensive.

When we find ourselves in a situation that we cannot handle effectively, threatening feelings are aroused, such as fear, anxiety, hostility, frustration, or other negative feelings. Inability to handle the situation is a threat to one‟s self-esteem. A possibility of physical or psychological harm may also exist. When there is a lack of competence for dealing with a situation, a defense mechanism may be used.

COMMON CHARACTERISTICS OF DEFENSE MECHANISMS

Unconscious: A person may be aware of what he/she is doing, but is unaware of the motivation.

They are defenses against anxiety; a threat is, by definition, anxiety arousing. One becomes anxious whenever he/she is threatened by evidence or implication of personal inadequacy or unworthiness of any sort.

Self-deceptive: Obviously, defense mechanisms are useful; they protect against loss of self-esteem in the face of defeat. They help us handle our feelings of discomfort, make it possible for us to “save face”, provide an apparently logical reason for the behavior used, and enable us to maintain self-respect in spite of the outcome. However, when they become *habitual devices* for avoiding reality, a person becomes a victim of his/her own motivation. *Defense mechanisms can help us, or they can become crutches that substitute for more effective ways of dealing with difficult situations.*

Defense mechanisms are impervious to logical attack.

All defense mechanisms involve partial or complete denial, distortion, or disguise of motives, acts, or thoughts that are self-belittling or self-degrading.
Many defensive behaviors are mixture of several conventionally labeled ones.

REPRESSION (Motivated Forgetting)
Repression is abrupt and involuntary removal from awareness of any threatening impulse, idea, or memory. With Freud‟s discovery of the unconscious came his discovery of repression, the most basic defense mechanism. *All other defense mechanisms grow out of repression.*

Freud frequently conceived of repression as the expression of a balance between the seeking after pleasure and the avoidance of “no pleasure”.

Primal Repression: The immediate result of primal repression is that the repressed idea or impulse is fixated or frozen in development.

Repression Proper: This second stage of repression is directed against any derivatives or associates of the originally repressed impulses that may enter consciousness. Repression proper is like an “after pressure”, to use Freud‟s descriptive phrase, whereby ideas, trains of thoughts, or perceptions that are associatively linked to the primal repressed impulse are also denied access to the consciousness. Repression proper thus cooperates with primal repression to ensure that unacceptable impulses and associated ideas remain out of conscious awareness.

In the case of the adolescent girl mentioned in the example above, who feels guilty about her sexual urges, the process of *primal repression* keeps these impulses from her awareness, and the *repression proper* keeps her boyfriend‟s name from consciousness, as he is the associative link to what she believes are unacceptable urges.

DENIAL (Motivated Negations)
Blocking external events from entry into awareness when perception of such stimuli is symbolically or associatively related to threatening impulses. Denial abolishes dangers “out there” by negating them, and is present to some degree in every defense mechanism.

Repression and denial are the most archaic defenses. Either of these is the prerequisite for any of the other defense mechanisms.

RATIONALIZATION
Rationalization is defined as hiding from ourselves the real reasons for our actions and

offering instead some self-justifying reason. This defensive strategy helps in preventing the admission of an inadequacy.

Although this defensive strategy can make so-called “failures” and “mistakes” more palatable, frequent use of rationalization is self-defeating because it can thwart efforts to improve one‟s competency.

PROJECTION (Displacement Outward)
Projection is the attribution of one‟s own unacceptable impulses, wishes or thoughts to another person or object. Using projection, these impulses become “ego-alien” as though they are not a part of the self. Another definition of projection could be „hiding unacceptable urges from ourselves by attributing them to other people‟.

DISPLACEMENT
Displacement is redirection of impulses, usually aggressive ones, onto a substitute target when the appropriate target is too threatening.

Displacement is an unhealthy method of using up creative energy.

TURNING AGAINST SELF (Self as Object or Target)
It implies redirection of impulses inwardly against oneself instead of outwardly toward the appropriate target. Usually results in masochistic feelings, inadequacy, guilt, and depression.

REACTION-FORMATION (Believing the Opposite)
This is the transformation of unacceptable impulses into their opposite and more acceptable forms: hate into love, love into hate.

REVERSAL (Active into Passive)
Similar to *reaction-formation*, reversal transforms an impulse from an active to a passive mode. Also similar to *turning-against-self.*

INTROJECTION (Taking Within)
Introjection is defined as incorporating into one‟s own behavior and beliefs the characteristics of some external object or admired person. The superego, for example, is an introjection of parents‟ values.

Introjections and Identification:
Although the terms *introjections* and *identification* have often been used synonymously, while they often accompany each other, it is possible to differentiate the two in a meaningful way in terms of the representational world. According to some psychoanalytic thinkers, so-called “early” introjection can be seen as the process of

building up the inner world of representations, a product of the "organizing activity of the ego." Introjection, of the sort, which is thought to result in the formation of an organized superego, can be regarded as the process of attributing certain (human and non-human) object representations a special status (e.g. a babysitter, a teddy bear, etc.), so that they are felt to have all the authority, power and safety of the real parents. The child then reacts, when his parents are absent, as if they were present. This does not mean that she/he copies them, for that would be a process of identification. *Identification* may be regarded as a change in the shape of a self-representation based on the model of another. One may, for instance, take on the identification of an organized group.

Projective Identification:
The most intensely studied type of explicitly interactional defensive activity was first described by psychoanalyst Melanie Klein (1882-1960): Imaginatively splitting off part of oneself and attributing it to another for the sake of controlling the other.

As a mode of communication, projective identification is one in which the subject makes him/herself understood by exerting pressure on another person to experience a set of feelings similar to his/her own. *Projective identification* can also be seen as a type of pathological symbiotic relationship in which the subject experiences the object of the projection as separate enough to serve as a receptacle for parts of the self, but sufficiently undifferentiated to maintain the illusion that one is literally sharing a given feeling with another person.

Denial is an essential component of *projective identification* and it profoundly affects an individual‟s representations of self and others. What the subject denies as some aspects of the self, s/he projects onto another. Clinical reports reveal that a wide range of unconscious contents, conflicts, and affects may be denied and then projected. When the object (person) is idealized, "good" parts of the self are projected; and when the object (person) is denigrated, "bad" parts of the self are projected.

IDENTIFICATION WITH THE AGGRESSOR (Adopting Feared Traits) Here, the subject adopts the traits or mannerisms of a feared person or object.

ALTRUISTIC SURRENDER (Sacrifice Self)
This is a form of projection in which a person his/her own needs vicariously by identifying with the satisfactions of another. In extreme form, a person may even give up his/her own ambitions to allow another‟s to be fulfilled.

SUBLIMATION (Acceptable Substitutes)
Sublimation is the transformation of an impulse into a socially productive and acceptable form.

Submerging oneself in his/her career in order to avoid intimacy is another example of sublimation. High achievers in our society, such as athletes, artists, surgeons, and other highly dedicated and skilled people, may be seen as reaching such high levels of accomplishment by directing other energies into their work. This is perhaps the best defensive strategy for a Physical Sexual during the *de-loving* process.

OVERCOMPENSATION
This is an attempt to cover up a personal weakness by focusing on another, more desirable trait.

UNDOING (Magical Cancellations)
This is a characteristic of an obsessive-compulsive personality, who performs magical gestures, or rituals, to cancel unacceptable thoughts, or acts, once the thought or act has been completed.
ISOLATION (Stripping of Emotion, Meaning)

As a characteristic of obsessive-compulsive neurosis, unacceptable impulses are retained in consciousness but divested of emotion and separated from connecting ideas to achieve an emotional-intellectual quarantine.

INTELLECTUALIZATION
This is synonymous with *Isolation.*

WITHDRAWAL
As a defense mechanism, withdrawal is either the shutting off of communication or the removal of oneself physically when a discussion becomes unpleasant and one refuses to participate further. With respect to the paradigm of Emotional and Physical Sexuality, the Emotional may frequently use this defensive strategy in an effort to avoid confrontation.

DAYDREAMING / FANTASY
This is a way to escape. It is a useful mechanism if it creates dissatisfaction with things as they are, inspires the setting of goals for the future (as in self-hypnosis or the *Mental Bank*), and leads to a course of action designed to attain these goals. Daydreaming can lead to invention or to inspiration for a creative activity. Daydreaming, however, can become more satisfying than life experiences. When habitually used, it provides an escape by substituting fantasy for reality, and results in maladaptive behaviors.

Suggestions: Fantasy often involves an image of only the end result or the achievement of a goal (e.g. a famous actor, or a scientific discovery). In this way the daydreamer is without the means to achieve his/her goals in real life. Hence, we must break these goals down for clients so their fantasies have more realistic qualities, i.e. the step by step process, that make goals seem more attainable.

SUBSTANCE DEPENDENCY (Mental Tricks)
This is another escape type of defense mechanism. The use of substances that have physical effects provides some relief from stress, or a diminished awareness of discomfort. Tobacco, alcohol, and other drugs all provide a form of escape.

ESCAPE INTO ILLNESS
This defensive strategy is common with children because they get attention from their parents when they are sick.

REGRESSION (Developmental Retreat)

This is a return to earlier modes of response when confronted with anxiety.

DISSOCIATIVE REACTIONS (Methods of Escape)
Post-traumatic stress can induce any of the following:

- Amnesia – Loss of memory.
- Fugue – “Fleeing” – when amnesia involves running away. A person may wander away from his/her home.
- Somnambulism – Sleepwalking.
- Dissociative Identity Disorder – Multiple Personality Disorder.

CHAPTER 13: - HANDWRITING ANALYSIS (GRAPHOLOGY)

Handwriting is a sub-conscious activity. It is an ideo-motor response that bypasses the critical mind and hence, this is one of the most important diagnostic tools. As a therapist, it helps you to get an inner profile of the client before you start talking with him/her.

SLANT \\\\ ||||| ////

- Slant is the protractor position of the letters in relationship to the line of the sentence.
- Letters that are written in a 90-degree angle to the base line of the sentence, i.e. straight up and down without any slant, is an indication the writer is head ruled. Makes decision on logic and reasoning.
- Letters slanted to the right indicate the writer will make decisions based on emotions, the degree of the emotion shown by the degree of the slant, i.e. from empathetic in a slight slant to hysterical indicated by the extreme slant.
- The writers who tend to have extroverted ability for anything to the right in handwriting are outgoing towards others.
- If the writing is to the left, then the writer is self-ruled. Anything to the left in writing is going to self. The writer is high emo and usually builds a wall around self. Insecurity is the reason of attacking people.
- The extreme could be psychotic.

ZONES≡≡≡≡≡

Upper Zone: The upper zone is indicated by the loops of letters such as “t”, “l” “f” “b” “h” “k” “d”. The upper zone deals with the writer‟s energy towards philosophical, spiritual and learning abilities.

Middle Zone: Any of the case letters indicate attitudes to everyday life. The middle loop of the “o”, for example, could show deceit. Look for other signs as well.

Lower Zone: The lower zone indicates the sexuality of the writer. The loops that are round, without corners or check-offs, indicate a physical sexual. The longer the loop, the higher the sexuality. The emotional sexual‟s loops will have points, or they will lack any loop whatsoever and will check-off. Again, the longer the lower stem, the higher the energy towards their sexuality. Follow through of the loops that do not reach the baseline show that the sexuality is incomplete.

Note: It takes more than one sign or trait to indicate a personality. Look for the majority.

THE LETTER "t"

The letter "t" indicates how the client wants people to see them (Ego).

- The T-Bar indicates goals.
- The higher the cross bar, the higher the goals.
- The lower the cross bar, the lower the goals.
- If the cross bar is at peak, this indicates a deep drive for success.
- If the bar starts strong and loses it, the writers energy is lost.
- If the bar goes in a downward fashion, it indicates anger.

THE LETTER "m"

- If the first two humps are perfectly round, the person is a 50-50.
- The more angular the humps, the more emotional.
- The rounder the humps, the more physical.

SIGNATURE

The signature is the logo of an individual. The signature shows the way the writer portrays oneself. Handwriting is how he actually is.

MARGINS

- If the margin is wide on the left, the client is general to self (emotional).
- If the left margin is narrow, they are more generous to others (physical).
- If all the paper is used, both margins, the person is very frugal. For this, also look to the bottom of the paper.
- If good top margin is given before writing it indicates writers respect to authority / elders / spouse

LARGE WRITING **LARGE WRITING**

Person having Large handwriting than the normal size would have a tendency to part with information easily and also is an easy person to give charge (Get hypnotized faster than usual).

SMALL WRITING **small writing**

- The person pays attention to detail.
- This person is very secretive and cautious.
- This person makes maximum utilization of available resources.

PRESSURE: **PRESSURE**

If the pressure is more then the person is physical
If the pressure is light then the person is emotional
Medium pressure is 50 : 50

MULTI SLANTS

If the writers words are having multi-slants in the same sentence repeatedly then the writer has mood swings.

If the writer has multi-slants with high energy variation then there is a possibility of disintegrated energy.

OTHER THOUGHTS

If the writing goes up, they are optimistic. If it goes down, they are pessimistic.

If the slant of the line goes straight down, the person is suicidal. Another sign of this is a line through the signature, a way of crossing themselves out. Falling words and letters indicate suicidal tendencies.

NOTE : BALANCING FACTOR IS ABSOLUTELY NECESSARY BEFORE POINTING A SUICIDAL TENDENCY.
DO NOT TELL YOUR PATIENT THAT THEY HAVE A SUICIDAL TENDENCY.
YOU NEED TO HAVE GOOD EXPERIENCE BEFORE MAKING ANY FRAGILE STATEMENT. IT CAN MAKE OR BREAK THE THOUGHT PATTERN OF ANY PATIENT.

Have the client write in BALL POINT PEN.
It is better to get a sample on unlined paper
Look for the angulations of the lines, and the way the words end.

ATTENTION

- *GRAPHOTHERAPY is an extremely powerful tool, Use it with responsibility*.
- The Above given factors can be used for Analysis only and **NOT FOR THERAPY**Every formation is different for different people, something that works for one may not work for the other
- Be well informed and equipped before using ANY of the healing tools.
- Your Patient has Faith in you so handle them with utmost responsibility and in an Empathetic

CHAPTER 14: - PAST LIFE REGRESSION

PAST LIFE REGRESSION

Introduction: Past life regression is based on the assumption or model of reincarnation. Such a model is considered a religious or metaphysical belief. As with all such beliefs, it is not the therapist‟s job to prove or disprove such beliefs, but rather to assist the client in exploring and discovering their own beliefs about the existence of past life experiences, and then to help such experiences create therapeutic movement within themselves.

Safer than Age Regression:

Past life regression, because it is once removed from the history of the client‟s present life, doesn‟t carry the fact/fantasy confusion problems present in age regression. For example, a client could experience sexual abuse in a past life experience, but they are not going to carry resentment for anyone around them in the current life.

Many therapists sidestep the issue of reincarnation and its validity by approaching the client strictly in the context of therapy. Therapeutically they assume that the past life experience, real or not, is rich in metaphoric expression of the emotions and unresolved conflicts within the client‟s subconscious mind. By seeking resolution of such conflicts, and the venting of such emotions in a past life experience, the benefit has the same impact as resolving or venting such issues in this life as well. Whether clients want to bridge the gap between issues in a past life is strictly up to them, and not required for the benefit to take place. In this way, past life regression is considered to work more within the framework of the client‟s defense mechanism than is age regression.

SAFEGUARDS

When using either age regression or past life regression certain safeguards should always be in place. One is the “hidden observer”. This technique allows the client to view past events as though they are just a hidden observer of the events, and not actually “there”. This provides the client with some protection from the emotional impact of the event. The person can experience even further removal from the impact with the suggestions that they are watching it on a TV screen. The sound can then be turned down, or the picture made smaller. The hidden observer technique can also allow the client to remain as their “adult self” and even have the “adult self” provide narration or make comments, suggestions, or give supportive advice to the “child self” in the event.

The hypnotist must remain in total control of the situation, watching the client carefully to judge when the client is experiencing too much anxiety. Flooding the client with

emotions can be anti-therapeutic, and should be avoided. By remaining in total control, the therapist can then instruct the client to “let go” whenever the emotions of a scene become too high.

The client could then be redirected to be right back in the therapist‟s office in the recliner chair feeling safe, comfortable and relaxed.

The “psychotic swing” or other signs of personality instability should be constantly watched for. If the hypnotist has any doubts about the stability of his client, he should not perform any regression until receiving a second opinion from a health professional trained to understand personality disorders. It is also advisable that the hypnotist does a good history, and suggestibility conditioning process, before regressing a client. Doing a regression in the first session is not usually advisable. Get to know your client better first.

CHAPTER 15: - THE MENTAL BANK

The Mental Bank is one of the most powerful tools as a hypnotherapist. It reinforces many types of therapies and speeds progress. It can be used for procrastination, motivation, goal attainment, prosperity, weight loss, smoking, etc.

As therapists, we should be using this program ourselves for two reasons: to understand how to use it so we can teach it to our clients, and to personally benefit from using it.

SYNERGISTIC APPROACH

The Mental Bank uses a synergistic approach, which means that the sum of the parts is more powerful than the parts individually. It is the result of Dr. Kappas" years of experience and research into reprogramming the subconscious mind.

RESISTANCE

The Mental Bank, like hypnosis, is a very powerful means of affecting the subconscious mind, so resistance is expected. The Mental Bank gets in touch with our subconscious limitations. It focuses us on our goals, and puts us in control of our lives. Resistance is a sign that we are running into our limiting programming and having an effect on it.
Welcome resistance; it is the first step towards change. It is important to help the client recognize and break through their resistance.

RECIPE FOR SUCCESS

There are five synergistic ingredients that make the Mental Bank powerful. They are:

Belief: We must understand and believe that we are successful right now! In other words, we are successfully acting out our script whether we are happy with it or not. If we"re not happy with it, we just need a better program. We will be just as successful fulfilling that one. We must get the client to understand this concept.

Daily Reinforcement: In order to override the old unwanted programs, the new one must be reinforced until it begins to take over. The Mental Bank is not a positive thinking program. Positive thinking deals with only the conscious mind, which is only 12% of our mind. That works for about 5% of the population, i.e., the natural somnambulists. The Mental Bank reaches 88% of the mind, or the subconscious mind. We do not have to believe that the Mental Bank will work for it to work. In fact, we can even think that it won"t work and it will still work, as long as we do it.

Scripting: Handwriting is an ideo-motor response that comes from the central nervous system. It also goes back into the central nervous system, bypassing the critical area of

mind, and thereby entering the subconscious mind. Printing is digital, and does not bypass the critical mind. Therefore, have the client hand write the script.

Time of Day: The last half hour before sleep is referred to as the magic 30 minutes. This is the time when a person"s mind is overloaded, and is in a natural state of hypnosis. Another important advantage is that something taken into the mind at this time goes into the precognitive stage of dreaming, instead of the venting stage.

Dreams: What we put into the Mental Bank during the magic 30 minutes goes into the wishful thinking stage, and then into the precognitive stage. The mind takes the data that was input during the magic 30 minutes, and then predicts where we are headed using this information.

The mind always wants to be prepared for what is coming up, so it begins to prepare us for success. Once we take in this new program, we begin to release the old program through the venting stage. With the Mental Bank, we take control of this very natural process of the mind.

REALITY v/s FANTASY

The subconscious mind does not know the difference between reality and fantasy. When we visualize or script something, the information goes into the subconscious as if it was real.

An interesting experiment has validated this theory. A researcher randomly divided a group of men into three groups. One group practiced shooting free throws for several weeks. Another group visualized shooting free throws, but did not practice. The last group didn"t practice or visualize. Not surprisingly, the last group made no improvement, in fact they got worse. The group that practiced shooting actual free throws for several weeks improved the greatest, but only slightly better than the group that only visualized shooting free throws. What we script or visualize becomes a known to the subconscious.

THEORY OF MIND

Understanding the Theory of Mind is the core of the Mental Bank concept.
Knowns represent emotional pleasure to the subconscious mind, and unknowns represent emotional pain. Our knowns become our comfort zone. Our comfort zone dictates a lot of our behavior.

For example: If our financial comfort zone is at Rs. 20,000 per month, anytime we go too much above that amount, we get anxious and begin to procrastinate or sabotage, in

an attempt to get ourselves back into our comfort zone. Anytime we go too much below that amount, we become motivated to get back into our comfort zone.

The Mental Bank is a way to consistently expand our comfort zone upwards.

Presuppositions: The Mental Bank uses two presuppositions as follows:

- You must accept that you are successful now. The mind is a good machine. You create your own reality, even if affected by negatives.
- You write the script, are part of the script, and can change the script. You are the star of your own movie. Now is the time to make your own rewrites. You know consciously where you want the rewrites to be. The subconscious is a goal machine. You will obtain all subconscious desires.

MENTAL BANK PROCESS

Contract:

We are going to make a contract with ourselves to rewrite our script. The amount of income should be based on our average yearly income. If you make Rs.100, 000 or less per annum, you double the amount of your income to set your Mental Bank goal.

If you make Rs.100, 000 or more, your goal should be four times the amount. Use the gross income amounts, not the net amounts. This amount will represent 25% of the total Mental Bank balance. The idea behind this is to live off the interest, not the principal.

To figure the hourly rate for value events, cross off the last three zeros from your Mental Bank goal. For example, an yearly Mental Bank goal of Rs.100, 000 would equal a rate of Rs.100 per hour.

Value Events:

Value events are the things we should be doing to achieve our goals. Choose them ahead of time, and pay yourself only for the ones that you have written down.

For balance, three categories are focused on when choosing value events: Happiness, Success, and Prosperity. If you want faster events in one area, then focus all your value events in that area.
Pick one or two goals for each category. Then come up with value events to accomplish those goals.

Value events are the positive things that we should be doing that are worth rewarding ourselves for.
Value events can be paid on an hourly basis, or on a per event basis.
Bonus events are the things that we should be doing but are more difficult for us. The

rate for bonus events would be double or more the standard value event amount. Jackpot events are one-off unexpected events which emphasize and work towards the basic goal. The rate for such an event could a pre-set amount.

Happenings:

The happening section is very important. Here we acknowledge happenings created by doing the Mental Bank and begin to focus on the positive things taking place. Through this, we reinforce the positive, and train the mind to look for positives. Happenings may not always appear positive on the surface.

Affirmations:

An affirmation is a positive thought that we want to become part of our script. These go in the investment section, or on a separate piece of paper. Pick one and use it for at least a week. Find the wording that produces the best results, and apply this to future affirmations.

Investments:

This section is used to get the person comfortable with making investments. We deduct money from the Mental Bank balance and make an imaginary investment in a stock, property, business etc. We follow the investment, and when we are ready to sell it, we take the money we invested, add any profit, and add it back into the Mental Bank balance. Once we have success with this process, we can begin to have confidence making investments with our real income.

CHAPTER 16: - REFERENCES

GENERAL SELF-IMPROVEMENT

OVERVIEW

For general self-improvement, you (the therapist) should be the guiding force and give the client direction. It is IMPORTANT that the client gets involved in his/her own therapy, and that they have positive thoughts about what they want to achieve.

Relationships: If the relationship has not gone too far down with too much hurt, work at saving the relationship. If too much damage has been done, work on getting the best out of the situation.

To assess the quality of a relationship look at the following:

- Like
- Trust
- Respect
- Good Communication
- Good Sex
- Similar Interests
- Separate Interests

When evaluating a person, men and women have different priorities in what is important

Priority	Men looking at women	Women looking at men
1	Looks	Success
2	Sexuality	Personality
3	Personality	Intelligence
4	Intelligence	Sexuality
5	Success	Looks

Career Direction: Work on improving the client‟s abilities, specially their communication abilities. GIVE THEM HOPE!

Sexual Behavior: Train them to get to know about their own sexuality. Make sure they feel neither over nor under sexed – just what is right for them.

Bad Memory

Teach the client the ability to make decisions, and to work with a “balance sheet”.

First Session
Give the client the sexuality questionnaire to bring back next time. Show it them, and explain that they are normal. Give suggestions for feelings of well-being and confidence.

Second Session
Dream Therapy: Go through the three stages of dreams. Enforce the idea, “You can‟t move forward while you hang on.”

Have them write an affirmation. Emotionals write, “I will vent out what is holding me back.” They must write this before going to bed. Have the client keep a diary about how they think and feel. One side of the page is feelings, while the other side is dreams.

For Weight Issues: Have the client keep a list of what they eat, when they eat, and how they were feeling when they ate. Next session, have them bring a list of what changes they would like. Give them confidence and esteem in hypnosis.

Third Session

Have the client bring back the lists and the dreams. Based on their inputs, give them nice, positive things to reinforce before going to sleep.

MEMORY IMPROVEMENT

Suggestion for the client: As you study, you will concentrate thoroughly, pulling up what you need when you need it.

Four steps to memory improvement: In hypnosis, teach the client:

- Relaxation
- Concentration
- Retain information
- Recall information

Outline a plan of how to work on it. Check the program each week to see where they are. The person must have a conscious desire for motivation to make changes.

How to Motivate

Have the client do the following:
Make a list of assets: Every good thing about who they are and what they are.

Write a description of the person that they would like to be (the kind of car, home, clothes etc.). Tell them, “You can assume you can be anyone you desire.” Concentrate on the mental image of the person you want to become. Feel the confidence and the courage of that person.

Physical Suggestibility, Physical Sexuality: They need success, so they can brag (in a good way) about what they have done. Give them suggestions to see themselves telling about how successful they are. Have them see themselves talking about their accomplishments.

Physical Suggestibility, Emotional Sexuality: They need to see themselves complimented on the success. Have them visualize it, sense it, and picture people telling them how good they are.

Emotional Suggestibility, Physical Sexuality: They need to get rewards. With them you need to set goals. As each goal is reached, set the next one.

Emotional Suggestibility, Emotional Sexuality: They are motivated by power, money, and control. Suggest to them. “Think about the power this gives you, the money it brings, and the control you have with that.” They keep escaping into a world of their own. Keep bringing them back.

Often, they will have a fear of success. A conscious desire for motivation must be developed. Teach them how to set goals, and have them visualize accomplishing those goals. Have them get in touch with how it feels.

RESISTANCE

Whenever there is a change in therapy, resistance occurs. This is a good thing to have happen. It means the therapy is working.

ATHLETICS

When working with athletics, have the client see the crowd. Desensitize them to the crowd. Make them realize that the crowd doesn‟t come there to bother them. Use ideomotor response. Let the crowd be there for the client. Have the clients see themselves winning, with the crowd cheering. Have them visualize the perfect performance, and get the feeling.

STRESS MANAGEMENT

Take a client deep and desensitize them. Block them to the negatives. With a Physical, give them invisible protection, blocking the negatives from coming in. The more the stress, the thicker the shield gets.

With an Emotional, tell them that the negatives continue to bounce off. Use a double bind. Examples are:

"The harder you try to lose, the better and more successful you become."
"The more you think about wanting a cigarette, the more you want to quit smoking."

Always in Hypnosis: Teach them about hypnosis. Teach them to stay out of hypnosis in the outside world, and just keep it for the therapy sessions. Teach them to count themselves out

Whenever they need to. Remember, they feel all the negatives because they are always in hypnosis.

MENTAL BANK

This is a very effective process to improve your level of happiness in life by changing your life script. Positive thinking alone does not change the subconscious mind. By using the Mental Bank, you create positives in three areas of your life:

- Happiness
- Prosperity
- Success in Relationships

YOUR BODY IS THE SHADOW OF YOUR MIND

Your Body is the shadow of your mind – can be used as reference points when conducting an EFT session

ANEMIA: Not recognizing one's self-value; a feeling of

weakness ARTHRITIS: Unable to express yourself, feeling

inadequate

CANCER: Severe hate and/or resentment toward another individual, but not expressing it openly; hate turned inward and carried throughout one‟s life

COLDS: Indecisiveness, need to make a decision and not doing so; feeling sorry for oneself and wishing to delay activity

COLITIS AND ELIMINATION PROBLEMS: Over attachment, holding in communication, not releasing situations

DIABETES: Holding back, a selfish attitude, not wanting to

give DIGESTIVE DISORDERS: Worry, not letting oneself

relax

EYE DISORDERS: Inability or refusal to see things as they really are

HEADACHES: Feeling threatened by one‟s environment, pressured

HEART ATTACK: Feeling pressured by responsibility, wanting to

escape HEART PROBLEMS: Lack of love

HEARING DISORDERS: Inability or refusal to listen or accept what is heard

KIDNEY DISORDERS: Condemnation of self; not forgiving self for something said or done, not understanding self-value, not accepting love

LOWER BACK PROBLEMS: Mental power struggle

LUNG DISORDERS: Restricted feeling, attitude of feeling loss of freedom MIDDLE BACK PROBLEMS: Apprehensive; fearful; unable to support

NECK & SHOULDER TENSION: Carrying other people‟s problems, feeling that you have the whole world on your shoulders

NERVOUS DISORDERS: Too demanding and restrictive of self, emotional build up

PROSTRATE DISORDERS (Genital): Not appreciating the masculine expression; guilt and / or fear in the expression of aggressive quality; feeling unproductive

REPRODUCTIVE DISORDERS (Female): Not appreciating the feminine expression; guilt and / or fear in the expression of receptive quality; feeling unproductive

SINUS PROBLEMS: Restricted in expression; feeling trapped, overly sensitive

STROKES: Problems with family of origin (Parent‟s family)

THROAT DISORDERS: Having to do with using the will; need to make decision and delaying in doing so, or not carrying through on decisions

ULCERS: Feeling out of control and wanting to control others and their situations.

MORE ON COUNSELING & INTERVIEWING

CREATIVE ILLNESS

Creative illness is the motivation for people to figure out what they do and why they do it. It is the foundation to build a theory. All great schools of psychology came from someone‟s creative illness.

FREUD

Freud was an analytical psychiatrist, and for 50 years was the accepted “god” in psychology. If you did not understand Freud, you understood nothing. Nothing else was recognized. The other two schools that surfaced later did so in rebellion.

Freud believed that childhood and early life had the greatest impact. He was brought up in the Victorian age. Sex at that time was repressed. He believed that early life experiences created the impact for the subconscious mind. This idea dominates most of psychology today.

Freud concluded that natural human urges were subject to molding by society which in turn created repressions and led to the establishment of defense mechanisms. He attempted using hypnosis to access the mind and assumed that everything he got out of the session was true. He believed that everything was linked to childhood and sex. Initially in a few cases, everything he postulated he found. However, in most of the cases the model failed and the medical profession would not accept this. So, the Freudian method came under a cloud.

Freud then came up with free association, looking for patterns. He was a high Emotional, obsessive compulsive, and worked in symbols. In therapy, no goals were set. There was no end to the therapy. It was just an exploration of self.

BEHAVIORISM

In the 1950‟s, therapists started dealing with people returning from wars, suffering from shell shock etc. This needed a new treatment. This developed a new school called Behaviorism, which means “Learning Theory”. It was a rejection of Freud in that it was classical and operant conditioning. This was in direct opposition to the “Little Black Box” theory.

This theory stated that the mind was like a little black box that should not be opened. Deal only with the observable behavior; don‟t ask anything. Observe and try to figure out goals to change behavior. The goal was not to just make the client feel better, but to make them understand that changes were the goal to feeling better.

B. F. SKINNER

Skinner was a very controlled person. He believed that everything happened with a controlled environment. He put his child in a box, and trained the child like one would train an animal. His feeling was that if the child were trained properly, then everything would be OK. He believed that everything we do is the result of environment.

PAVLOV

Pavlov was studying the digestive juices of dogs and their functioning. He determined that there were two types of responses:

Unconditioned response, which is natural; and conditioned response, which is brought on by training and conditioning.

90% of NLP is taken from this.

OPRANT CONDITIONING

The stimulus creates a response, which may be either positive or negative. If the response is positive, then the reward is greater. If the response is negative, then the reward is less.

TABULA-ROSA

Tabula-Rosa means “blank slate”. Freud thought that we started out as horny little miserable things. The behaviorist thought that we start out as a blank slate.

Analysts thought that schizophrenia was an undeveloped superego. The behaviorists thought that craziness was reinforced, and that people were rewarded for their craziness by the environment. Therefore, the environment had to be created for proper behavior.

COGNITIVE BEHAVIORISM

Cognitive behaviorists want to open the little black box.

Definition from Psychiatric Dictionary:

“An active, structured, time-limited, and directive form of therapy, based on the belief that the way a person perceives and structures the world determines his feelings and behavior. Depression, for example, is an outgrowth of a tendency to view oneself in a

negative way, and treatment is aimed at altering that cognitive schema by helping the client gather evidence for and against his distorted self-view."

HUMANISM

Definition from Psychiatric Dictionary:

"An orientation that rejects both quantitative reductionism of behaviorism and the psychoanalytic emphasis on the unconscious forces in favor of a view of man as uniquely creative and controlled by his own values and choices. Through experimental means, each person can develop his greatest potential, or self-actualization. Humanistic Psychology is related to the human potential movement and its encounter groups, growth centers, sensitivity training, etc."
There are two different approaches to humanistic psychology:

Carl Rogers:
Rogers" therapy is based on what you need to know. Randomly, everything they need to say has a purpose. A Rogerian therapist does not speak With practice, you are then able to make calculated moves.

The goal is to give unconditional positive regard. This is manifested in loving, warm acceptance for their feelings, whatever they are. Never give advice or answer questions. Never make interpretations or ask questions.

Help the client explore his/her feelings. Content vs. Feelings: Content is totally unimportant to a Rogerian therapist. The goal is to help explore the different feelings they have. They believe that feelings are painful, and we dam up the feelings because they hurt. The goal is, therefore, to un-dam the feelings. A Rogerian therapist is totally equal to the client, and never one-ups the client. He really listens to what the client says.

Fritz Pearl:
Pearl"s theory is very directive, and uses transactional analysis. He is more concerned with content and less concerned with feelings.

CODE OF CONDUCT FOR ALL STUDENTS / THERAPISTS

1. We are primarily learning Integrated Clinical Hypnotherapy for Self-focus and Self-Empowerment.

2. We are the Lords and Masters of our Universe. Our Universe is a manifestation / projection, consciously or unconsciously of ourselves.

3. Therapy starts with ourselves and we do not blame others or situations for what we manifest in our Universe.

4. We will only attract the clients that we are ready for and are eligible to handle.

5. We inform clients adequately about the therapy and its limits.

6. We do not misrepresent our professional qualifications, affiliations, and purposes. We disclose, upon request, our training, qualifications, and competencies to prospective clients.

7. We do not reject clients on the basis of gender, race, ethnic background, culture or sexual preference.

8. Our reception and our therapy room are appropriate. Our personal hygiene and the hygiene of our therapy room are impeccable.

9. We are responsible for the therapy, but never act against the best interest of our clients, or against their express will. However, we cannot shirk responsibility by claiming to follow a request from a client. The welfare of the client is the prime consideration at all times. When the client‟s desires and our professional responsibility conflict, we will end the therapeutic relationship courteously. We do neither blame the client nor ourselves for that.

10. As we deal with intimate thoughts, feelings and details of our client‟s life, we retain a professional distance. We present our work and ourselves professionally. We do not behave as personal friends or acquaintances. We do not engage in intimate relationships of any kind with clients.

11. We recognize transference, counter – transference and other projections, and seek to restrain these. We don‟t assume the role of Helper or Savior; we discourage admiration and deference from the client. We avoid making clients dependent on us; we stimulate their own responsibility and independence.

12. We do not try to convert clients to any belief or practice. We respect their convictions, though we may point out how these seem to relate to their problems, if this seems the case.

13. We build a relationship of trust with the client and will not betray that trust. We will treat all information from clients confidentially and will not divulge it without the full consent of the client, except when obliged by law to do so. In publications we protect the anonymity of the client or ask written permission.

14. We only perform those therapies for which we are qualified. Unless otherwise licensed to do so, we do not provide or offer to provide diagnosis or treatment which is limited by law to appropriately licensed practitioners.

15. We stay alert if clients need anything outside our own experience or training. We then refer to colleagues or suggest other suitable methods outside our field.

16. We advise clients to see a doctor when problems could have medical cause. We never suggest decreasing or stopping medication. We limit ourselves to psychosomatic diseases and physical complaints for which no physical cause has been found. We do not pretend to heal serious and terminal diseases, but at best to influence favorably the course of such diseases.

17. Sessions are only recorded with the consent of the client. The client may withdraw his consent rights after the session. Clients are entitled to copies of records of sessions.

18. We refrain from making disparaging remarks to clients about colleagues or other therapies in the regular or alternative field.

19. We respect and safeguard the professional works and original ideas of others, and give full credit and appropriate citation for such works and ideas when used in our publications and presentations.

20. We are willing to discuss our professional activities with fellow members. We collaborate with complaint procedures.

21. We will assess our own competence, skills and limitations. We seek additional knowledge and skills through participation in seminars. Workshops or trainings and other opportunities. We keep informed about developments in the profession and in society.

22. We will collaborate with any serious research that does not prejudice the therapy process. We adhere to the generally accepted academic guidelines in conducting research with people.

23. We exercise prudence and discretion in promoting, encouraging, and facilitating public awareness and understanding of the field. We avoid sensationalism, inaccuracy or misleading claims.

24. We adhere to professionally accepted guidelines and standards of good taste in informing the general public of the availability of our services.

25. We seek to protect the public from all those who are not qualified to practice by deficient education, preparation, competence, emotional stability, or ethical standards.

26. When we observe inappropriate professional practices of fellow members, we will, whenever possible, discuss this with them. If this does not work, we will inform EKAA in writing.